

IN THE UNITED STATES DISTRICT COURT
FOR THE DISTRICT OF COLORADO

FILED
UNITED STATES DISTRICT COURT
DENVER, COLORADO

AUG 28 2014

Civil Action No. _____

(To be supplied by the court)

JEFFREY P. COLWELL
CLERK

Aaron S. Rabidue

, Plaintiff,

v.

JURY TRIAL DEMAND

Trudy Sicotte, in her official capacity as Medical Provider for the Colorado Department of Corrections-Limon Corr. Facility, and in her individual capacity;

Nicole Blatnick, in her official capacity as Health Service Admin. for the Colorado Department of Corrections-Limon Corr. Facility, and in her individual capacity;

Tina Rosler, in her official capacity as a Medical Nurse for the Colorado Department of Corrections-Limon Corr. Facility and in her individual capacity;

Alvin Massenburg, in his official capacity as Medical Provider for the Colorado Department of Corrections-Limon Corr. Facility, and in his individual capacity;

Martinez, in his official capacity as Chief Medical Officer for the Colorado Department of Corrections, and his individual capacity;

Jennifer Novatny, in her official capacity as in a Supervisory Position with Pharmacy for the Colorado Department of Corrections, and in her individual capacity;

John Doe, in his official capacity as Deputy Director of Prisons, Clinical Services for the Colorado Department of Corrections, and in his individual capacity;

Mr. Long, in his official capacity as, Defendant(s).

a Major for the Colorado Department of Corrections-Limon Corr. Facility, and in his individual capacity;
(List each named defendant on a separate line.)

PRISONER COMPLAINT

~~REDACTED~~

Ms. Falk, in her official capacity as a Warden for the Limon Correctional Facility - Colorado Department of Corrections, and in her individual capacity;

Hong Dang, in her official capacity as a Medical nurse for the Limon Correctional Facility - Colorado Department of Corrections, and in her individual capacity;

Rick Raemisch, in his official capacity as Executive Director for the Colorado Department of Corrections, and in his individual capacity.

A. PARTIES

1. Aaron S. Rabidue #154691, Limon Correctional Facility
(Plaintiff's name, prisoner identification number, and complete mailing address)
49030 State Hwy 71 South, Limon, Colorado 80826-0001

2. Trudy Sicotte, (Individual and Official Capacity) Medical Provider
(Name, title, and address of first defendant)
Limon Correctional Facility, 49030 State Hwy 71, Limon, CO. 80826
At the time the claim(s) alleged in this complaint arose, was this defendant acting under
color of state law? xxYes ___ No (CHECK ONE). Briefly explain your answer:
At the time this claim arose, Defendant was working for the
Department and is mentioned within this claim.

3. Nicole Blatnick, (Individual and Official Capacity) Health Service
(Name, title, and address of second defendant) Administrator
Limon Correctional Facility, 49030 State Hwy 71, Limon, CO. 80826
At the time the claim(s) alleged in this complaint arose, was this defendant acting under
color of state law? xxYes ___ No (CHECK ONE). Briefly explain your answer:
At the time this claim arose, Defendant was working for the
Department and is mentioned within this claim.

4. Tina Rosler, (Individual and Official Capacity) Medical Nurse
(Name, title, and address of third defendant)
Limon Correctional Facility, 49030 State Hwy 71, Limon, CO. 80826
At the time the claim(s) alleged in this complaint arose, was this defendant acting under
color of state law? xxYes ___ No (CHECK ONE). Briefly explain your answer:
At the time this claim arose, Defendant was working for the
Department and is mentioned within this claim.

(If you are suing more than three defendants, use extra paper to provide the information requested above for each additional defendant. The information about additional defendants should be labeled "A. PARTIES.")

A. PARTIES

5. Alvin Massenburg, (individual and official Capacity), Medical Provider
Limon Correctional Facility, 49030 State Hwy 71, Limon, CO. 80826
At the time the claim arose in this complaint, this Defendant was working under the color of state law and the Defendant was working for the Department when this claim arose and the Defendant is mentioned within this complaint.
6. Martinez, (Individual and Official Capacity), Chief Medical Doctor
Colorado Department of Corrections, 2862 South Circle Drive, Colorado Springs, CO. 80906
At the time the claim arose in this complaint, this Defendant was working under the color of state law and the Defendant was working for the Department when this claim arose and the Defendant is mentioned within this complaint.
7. Jennifer Novatny, (Individual and Official Capacity), Supervisor in Pharmacy
Colorado Department of Corrections, 2862 South Circle Drive, Colorado Springs, CO. 80906
At the time the claim arose in this complaint, this Defendant was working under the color of state law and the Defendant was working for the Department when this claim arose and the Defendant is mentioned within this complaint.
8. John Doe, (Individual and Official Capacity), Deputy Director of Clinical Services
Colorado Department of Corrections, 2862 South Circle Drive, Colorado Springs, CO. 80906
At the time the claim arose in this complaint, this Defendant was working under the color of state law and the Defendant was working for the Department when this claim arose and the Defendant is indirectly involved within this complaint as a policy and procedure maker.
9. Long, (Individual and Official Capacity), Major - Limon Correctional Facility
Limon Correctional Facility, 49030 State Hwy 71, Limon, CO. 80826
At the time the claim arose in this complaint, this Defendant was working under the color of state law and the Defendant was working for the Department when this claim arose and the Defendant is mentioned within this complaint.
10. Falk, (Individual and Official Capacity), Warden - Limon Correctional Facility
Limon Correctional Facility, 49030 State Hwy 71, Limon, CO. 80826
At the time the claim arose in this complaint, this Defendant was working under the color of state law and the Defendant was working for the Department when this claim arose and the Defendant is mentioned within this complaint.
11. Dang, (Individual and Official Capacity), Nurse - Limon Correctional Facility
Limon Correctional Facility, 49030 State Hwy 71, Limon, CO. 80826
At the time the claim arose in this complaint, this Defendant was working under the color of state law and the Defendant was working for the Department when this claim arose and the Defendant is mentioned within this complaint.
12. Rick Raemisch, (Individual and Official Capacity), Executive Director - CDOC
Colorado Department of Corrections, 2862 South Circle Drive, Colorado Springs, CO. 80906
At the time the claim arose in this complaint, this Defendant was working under the color of state law and the Defendant was working for the Department when this claim arose and the Defendant is indirectly involved within this complaint as a policy and procedure maker.

B. JURISDICTION

1. I assert jurisdiction over my civil rights claim(s) pursuant to: (check one if applicable)

xxxx 28 U.S.C. § 1343 and 42 U.S.C. § 1983 (state prisoners)

____ 28 U.S.C. § 1331 and *Bivens v. Six Unknown Named Agents of Fed. Bureau of Narcotics*, 403 U.S. 388 (1971) (federal prisoners)

2. I assert jurisdiction pursuant to the following additional or alternative statutes (if any):

28 U.S.C. 1331, 28 U.S.C. 1343, 28 U.S.C. 1367, 42 U.S.C. 1983

28 U.S.C. 1391(b)(2) because Colorado is where the events occurred.

C. NATURE OF THE CASE

BRIEFLY state the background of your case. If more space is needed to describe the nature of the case, use extra paper to complete this section. The additional allegations regarding the nature of the case should be labeled "C. NATURE OF THE CASE."

The Plaintiff in this 42 U.S.C. 1983 is incarcerated within the Colorado Department of Corrections at the Limon Correctional Facility in Limon Colorado. The Plaintiff asserts that the named Defendant(s) are **NOT** providing him with "reasonable adequate" medical care, a care that is at a level reasonably commensurate with modern medical science and of a quality acceptable within prudent professional standards, and designed to meet routine and emergency medical care. The Plaintiff in this complaint suffers from a traumatic 2006 spinal cord injury from being stabbed in the back and this injury has left the Plaintiff with neuropathic pain and his **Left Foot and Leg** has suffered a mobility loss and decrease in his **Left** calf muscle which also contains atrophy. The Defendant(s) named in this complaint refuse to provide the Plaintiff with "a continuous and effective" pain medication to treat the neurological nerve damage that inflicts muscle pain upon this Plaintiff and these Defendant(s) refuse to provide the Plaintiff with physical therapy which was recommended for treatment.

The Plaintiff on 01/31/2014 was seen within medical at the Limon Correctional Facility due to the Plaintiff declaring a medical emergency, because the Plaintiff started having severe pain within his **Left** foot. The Defendant(s) in this complaint treated the Plaintiff's **Left** foot pain as if he had a sprain, strain, or muscle ache when in fact the Plaintiff's **Left** foot contained a 5th metatarsal fracture. It took the Defendant(s) **six (6) days** to get the Plaintiff's **Left** foot x-rayed and then another **nine (9) days** to the x-ray results back. Therefore, the Plaintiff had to walk around with a fractured **Left** foot with **NO** crutches, **NO** pain medication, his foot wrapped (stabilized) in a ace wrap, and had to continue to walk up a flight of stairs, because the Defendant(s) would not give him a second

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(2nd) tier housing restriction. These Defendant(s) showed a deliberate indifference to the Plaintiff's medical needs and subjected this Plaintiff to much unnecessary pain and suffering.

On 03/04/2014, the Plaintiff declared another medical emergency, because when the Plaintiff woke up and went to stand up that morning, the Plaintiff heard a pop or cracking sound that sounded like a bone breaking and then the Plaintiff felt some severe pain within his left foot. The Plaintiff on this day was NOT given any effective pain medication, the Plaintiff's left foot was NOT stabilized, was NOT given a pair of crutches to assist him in walking and or to prevent him from applying any sort of weight on this left foot that would prevent any further injury. The Plaintiff's left foot was only wrapped in a ace wrap. The Plaintiff was NOT given a second (2nd) tier housing restriction to keep him walking up the stairs to his cell which could cause more possible injury. The Defendant(s) did order for the Plaintiff's left foot to be x-rayed again, but this x-ray was not taken until the following day of 03/05/2014, but the Defendant(s) did NOT get the x-ray results on this same day.

On 03/11/2014, the Plaintiff was called into the medical department and the Defendant(s) told the Plaintiff that his left foot now sustained additional fractures. On this date the Defendant(s) told the Plaintiff that he had additional fractures now at the 2nd and 4th metatarsals. The Plaintiff would like to point out that it took these Defendant(s) six (6) days to get the x-ray results again. These Defendant(s) showed a deliberate indifference to the Plaintiff's medical needs and subjected this Plaintiff to pain and suffering. The Plaintiff asserts this is not adequate medical care, because it should NOT taken these Defendant(s) this long to recieve the results on any x-ray and it should not taken these Defendant(s) as long as it did to have this Plaintiff's left foot injury x-rayed.

On 04/14/2014, the Plaintiff declared another medical emergency for his left foot due to severe pain that he was enduring within his left foot. This time when the Plaintiff stood up his left foot bent inward and the Plaintiff recieved a severe amount of pain. On this day, the Plaintiff requested for the Defendant(s) to have his left foot x-rayed again, because the Plaintiff felt that he had endured another injury to his left foot. The Defendant(s) denied the Plaintiff's request to have his left foot x-rayed again. The Plaintiff was given six (6) packages of Tylenol and was told that he had an upcoming appointment to see a foot specialist. The Plaintiff was NOT given any crutches to assist him in walking or to assist him in minimizing the pressure that the Plaintiff had to apply on his left foot in the process of walking or when the Plaintiff had to walk up the stairs to the second (2nd) tier to go to his cell and the Plaintiff was NOT given an effective pain medication to relieve the pain that the Plaintiff was having within his left foot.

The Plaintiff would like point out for this Honorable Court that there was several times when the Plaintiff went into the medical department at the Limon Correctional Facility when he declared these medical emergencies for the pain that the Plaintiff was enduring within his left foot and when in fact the Defendant(s) already knew that the Plaintiff's left foot several fractures. The Defendant(s) like Blatnick, Rosler, and Dang would take Defendant Massenburg the Plaintiff's medical chart and records contained within that medical file, but yet Defendant Massenburg would NOT give the Plaintiff a physical exam and Defendant Massenburg would NOT even come and speak with the PLaintiff himself so he could make a accurate determination pertaining to the Plaintiff's medical condition. The Plaintiff asserts that

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this is **NOT** a professional standard for adequate medical care and by Defendant Massenburg **NOT** giving the Plaintiff a physical exam when in fact Defendant Massenburg knew that the Plaintiff was having complications with his **left foot** recieving an unexplained reason for obtaining fractures. The Plaintiff states that Defendant Massenburg showed a deliberate indifference to his medical needs, cared less about the true medical state of the Plaintiff and subjected this Plaintiff to pain and suffering which all could have been avoided.

On 04/17/2014, the Plaintiff was transported to the Denver Health Center in Denver Colorado to be seen by a foot specialist. This foot specialist performed a physical exam on the Plaintiff's **left foot** and also conducted several x-rays on the Plaintiff's **left foot**. The x-ray report revealed that the Plaintiff **did in fact recieve more additional fractures** than the Plaintiff already had in his **left foot**. The x-ray that was taken on 04/17/2014 showed that the Plaintiff had **additional fractures of the metatarsal 2 through 5 on the left foot**. This documentation is found within **Exhibit #5 - Document #2**.

The Plaintiff asserts that on this date of 04/17/2014 this foot specialist stabilized the Plaintiff's **left foot** so it could **NOT** move. The Plaintiff's **left foot** was placed in a Cam walker boot and was ordered to wear this when he walks. This foot specialist stated that if they immobilize the patient's ankle to reduce tendon motion over the metatarsal through the toes and place the Plaintiff in a rocker bottom type piece of foot gear, then the Plaintiff will **NOT** be able to bend his toes at the metatarsophalangeal joints and this will likely allow the Plaintiff to heal more effectively.

As the Plaintiff stated supra, these Defendant(s) did **NOT** seek to immobilize his **left foot** so he could **NOT** move it and these Defendant(s) did **NOT** give him a second (2nd) tier housing restriction. It is only common sense and one that any doctor like Defendant Massenburg should know, when a person walks up a flight of stairs the tendons and the bones within the foot will flex and move in the contour motion the foot proceeds through as it travels up a flight of stairs. The Plaintiff asserts that if Defendant Massenburg would have properly immobilized his foot and given him a second (2nd) tier housing restriction that would have made security place the Plaintiff in a first (1st) floor cell. Then the Plaintiff would **NOT** continued to receive additional fractures and would **NOT** had to suffer from the pain and suffering that these Defendant(s) subjected this Plaintiff to. These Defendant(s) showed a deliberate indifference to Plaintiff's medical needs, afflicted unnecessary pain and suffering upon this Plaintiff.

The Plaintiff had to endure **three (3) months of pain and suffering and multiple fractures within his left foot before these Defendant(s) got the Plaintiff to a foot specialist to recieve adequate medical care**.

When the foot specialist at the Denver Health Center x-rayed the Plaintiff's **left foot** the x-ray revealed that the Plaintiff had a **new fracture at the 3rd metatarsal, various stages of healing, 2nd through the 5th metatarsal, but the 3rd metatarsal was a completely new fracture**.

These Defendant(s) also showed a deliberate indifference to the Plaintiff's medical needs by **NOT** giving this Plaintiff any medication for pain. The Defendant(s) gave the Plaintiff several excuses as to why they could **NOT** give this Plaintiff a "continuous and effective" pain medication.

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The excuse that the Defendant(s) gave this Plaintiff are:

1. The chief medical doctor will **NOT** let them perscribe Naproxin or Salsalate for a period of more than 14 days. Stating that long term use can affect the kidneys, liver, and other organs. The Plaintiff showed them that even though this fact could be true, it is also a proven medical fact that those symptoms as those stated above and that of stomach ulcers can be monitored though laboratory blood work.
2. The Defendant(s) also told the Plaintiff that they can **NOT** prescribe Naproxin or Salsalate for more that 14 days, because due to a new distribution policy within the pharmacy.
3. One particular Defendant Blatnick in a meeting with the Plaintiff told the Plaintiff that the distribution of pain medication(s) was done on the grounds of budget cut, cost cutting reasons.
4. The Defendant(s) would give the Plaintiff Tylenol 3, but would only do so for no more than 3 days and when the Plaintiff inquired into this reason, Defendant Blatnick said that in order for them to prescribe the Plaintiff Tylenol 3 for more than 3 days, That they (Defendant Blatnick and or Massenburg) had to get approval from the Limon Correctional Facility administration. The Plaintiff asked how could administration personnel approve such approval concerning a medical issue when those within the prison adminstration are **NOT** medically licensed or that of medical practitioners. Defendant Blatnick said that is the policy and procedure that is conducted here at the Limon Correctional Facility. The Plaintiff learned that these individuals within adminstration that are making medical approvals concerning medication are Defendant Falk and Defendant Long.

Let the Plaintiff assert that he has showed Defendant Blatnick in case law that was provided to him through another inmate that cutting pain medication on the grounds of cost creates a deliberate indifference to his medical needs. Defendant Blatnick asserts that she is only following the orders or policies that were passed down to her by the Chief Medical Doctor, the Pharmacy Supervisor, and that of administration. The Plaintiff further showed her according to the law, Just because you are following the orders haned down to you does **NOT** mean that you cannot be found liable when you know that the orders that you are following are violating a U.S. Constitutional Right and are showing a deliberate indifference to ones medical needs.

Defendant Blatnick is the Health Service Administrator at the Limon Correctional Facility and she serves as "Gatekeeper" and her role in this claim was to serve as gate keeper and **NOT** allow the Plaintiff to endure the pain and suffering that he did and should **Not** have allowed the actions of deliberate indifference to his medical needs occur. Defendant Blatnick did **NOT** fulfill her role as "gatekeeper".

(Sealock, 218 F.3 at 1209 (10th Cir. 2000)(a prison medical official who know that her role in a particular medical scenario is solely to serve as gatekeeper to other medical or prison personnel may be held liable under deliberate indifference if that person "dealays or refuses to fulfill that gatekeeper role).

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As the Plaintiff has showed this Honorable Court in that above, the Plaintiff did in fact sustain additional fractures in his **left foot** by the Defendant(s) named within this claim. The Defendant(s) in this Complaint did **Not** provide the Plaintiff with "reasonable adequate" medical care, a care that is at a level reasonably commensurate with modern medical science and a quality acceptable within prudent professional standards, and designed to meet routine and emergency medical care. The Plaintiff in this Complaint met with the Defendant(s) at various times which the Plaintiff will outline in the following "Chronology of Events" and will show this Honorable Court that the Defendant(s) showed a deliberate indifference to his medical needs and subjected this Plaintiff to a severe amount of unnecessary pain and suffering, violating his 8th and 14th Amendment Rights of the United States Constitution.

Walking, standing, lifting, bending, and working are considered activities that are a central importance to daily life. The Plaintiff has asserted within his medical records and within this Complaint that the pain he receives from his 2006 stabbing injury, the neuropathic pain is affecting his ability to walk properly, his ability to stand, lift, work, and the pain affects his ability to get any proper sleep. These are Major Life Activities that have been affected by the named Defendant(s) and are still to this day continuing to be affected by these Defendant(s). Major Life Activities are outlined in 29 CFR 1630.2 (h)(2)(j); 29 CFR 1630.2 (j)(1).

The Plaintiff asserts that these Defendant(s) will claim that the Plaintiff was seen by medical on numerous occasions which they have stated in the grievances that the Plaintiff has filed, but the Plaintiff states that the "gravamen" of his medical problem was **NOT** met.

(Sulton v. Wright, 265 F. Supp. 2d 292, 300 (S.D.N.Y. 2003) ("even if an inmate receives extensive medical care, a claim is stated if...the gravamen of his problem is not addressed"); Lavendar v. Lampert, 242 F. Supp. 2d 821, 843 (D. Or. 2002) ("deliberate indifference claim was supported where the Plaintiff was examined regularly by medical staff but "there is an ongoing pattern of ignoring, and failing to timely respond to or effectively manage, Plaintiff's chronic pain").

The Plaintiff stated supra how the Defendant(s) would **NOT** provide him with a "continuous and effective" pain medication. The Plaintiff will show this Honorable Court how these Defendant(s) directed the Plaintiff that he can purchase pain medication from canteen. This is further proof of the deliberate indifference that the Defendant(s) showed the Plaintiff's medical needs, the continuation of pain and suffering inflicted on this Plaintiff when these Defendant(s) knew that this Plaintiff was in pain.

The Supreme Court has stated that "deliberate indifference to serious medical needs of prisoners constitutes the 'unnecessary and wanton infliction of pain'...proscribed by the Eighth Amendment".

(Estelle v. Gamble, 429 U.S. 97, 104, 97 S. Ct. 285 (1976))

*** SEE THE FOLLOWING CHRONOLOGY ***

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18th of September 2012; The Plaintiff on this date was seen within the medical department at the Limon Correctional Facility and was seen by the medical provider Defendant Sicotte. The Plaintiff was seen by Defendant Sicotte on this date for the pain that he was enduring within his back and legs. The pain that this Plaintiff is having comes from a stabbing assault back into 2006 when the Plaintiff was living in California. (See the Plaintiff's California medical records, Exhibit #3) The Plaintiff on this date was in pain and went to medical, because he submitted a medical request kite to be seen by medical for this pain. The Plaintiff wanted to be placed back on his pain medication which included Neurotin, Baclofen, and Narco. Defendant Sicotte refused to place this Plaintiff on these medications or even prescribe him a adequate pain medication to relieve this pain that the Plaintiff was and still is suffering from. Defendant Sicotte instead gave the Plaintiff a 60 day prescription for Ibuprofen and dismissed the Plaintiff from the medical department. Defendant Sicotte did have the Plaintiff fill out a medical record request form so the medical department at the Limon Correctional Facility could recieve the medical records from California for this 2006 stabbing. The Plaintiff did fill the medical records request form out and did give it right back to Defendant Sicotte. Let the Plaintiff state that the stabbing to the Plaintiff's back was a stabbing to the Plaintiff's spinal cord. Let the Plaintiff's medical document Exhibit #1 - Document #3 reflect that this Plaintiff has a documentation within his medical file that state, "338.21 - Chronic Pain Due To Trauma".

8th of November 2012; The Plaintiff on this date was seen by Defendant Sicotte within the medical department. The Plaintiff was there in medical to request being placed back on Neurotin which is a pain medication to treat neuropathic pain, nerve damage pain from trauma. The Plaintiff told Defendant Sicotte that he has nerve damage in his left leg, but Defendant Sicotte lists the nerve damage bieng in that of the right leg. Further exhibits within this complaint will show many more medical documentation errors on behalf of Defendant Sicotte. On this date Defendant Sicotte list medical facts onto Exhibit #1 - Document #4 and these medical facts are medical recorded facts that was taken from the Plaintiff's California medical records, Exhibit #3. This is proof that Defendant Sicotte and the medical department at the Limon Correctional Facility did recieve the medical records from California. Defendant Sicotte on Exhibit #1 - Document #4 list that the Plaintiff has some atrophy of R calf. Here the Defendant Sicotte states that the Plaintiff has atrophy in the right calf when in fact this atrophy is in the Left calf and the Plaintiff will show that further within this complaint as well. Defendant Sicotte states that the Plaintiff cannot completely pivot the left ankle and does walk with out a limp. On this day that the Plaintiff saw Defendant Sicotte. Exhibit #1 - Document #4, states that the Plaintiff has "338.21 - Chronic Pain Due To Trauma". As this Exhibit #1 - Document #4 will reflect, when the Plaintiff left the medical department, Defendant Sicotte did NOT prescribe this Plaintiff any medication for pain. Defendant Sicotte state that the Plaintiff performs two hours of strenous exercise a day. This is NOT what the Plaintiff told Defendant Sicotte. The Plaintiff told Defendant Sicotte that he does routine stretches throught-out the day.

Defendant Sicotte did submit a Non-Formulary Drug Request for the Plaintiff for the medication - Neurotin, Exhibit #1 - Document #5. Defendant Sicotte states on this document that the Plaintiff has atrophy of L calf. This is correct! Let this Honorable Court see that Defendant Sicotte states on Exhibit #1 - Document #4 that the Plaintiff has atrophy in the Right calf.

Defendant Sicotte on this day further submitted Exhibit #1 - Document #7 for the Plaintiff to recieve physical therapy. The Plaintiff would like this Honorable Court to see that on this document Defendant Sicotte hand wrote that the Plaintiff has atrophy in the R calf. This is a request for the Plaintiff to recieve physical therapy, but as this Honorable Court can see. Defendant Sicotte compromised this approval for the Plaintiff to recieve physical therapy, because Defendant Sicotte did not state medical facts. Defendant Sicotte falsified the request for physical therapy by contradicting her medical findings.

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The Plaintiff would like this Honorable Court to look at **Exhibit #1 - Document #8** which states the Plaintiff's physical therapy has been denied. The Plaintiff states that if Defendant Sicotte would have included the true medical facts within this request then the Plaintiff might have been granted to receive physical therapy.

The Plaintiff would like to show this Honorable Court another documentation Defendant Sicotte listed on **Exhibit #1 - Document #5** that is **incorrect**, another falsified medical fact. On **Exhibit #1 - Document #5**, Defendant Sicotte states that the Plaintiff is on **NO MEDICATIONS**, "I/M is requesting Neurotin and will **NOT** take anything else after talking to him". The Plaintiff would like to point out that he **WAS ON 600mg of IBUPROFEN**, a medication that was prescribed to the Plaintiff by Defendant sicotte, **Exhibit #1 - Document #6**. These documents, **Exhibit #1 - Document(s) #4, #5, #6, #7, and #8**, were all created on the same day. The Plaintiff states that Defendant Sicotte's intentions were **NOT TO TRULY HELP HIM**, but rather Defendant Sicotte acted or treated the Plaintiff as if he was a nuisance, showed **NO** true concern for this Plaintiff's health and subjected this Plaintiff to much pain and suffering. Showing a deliberate indifference to the medical needs of the Plaintiff.

30th of December 2012; The Plaintiff filed his Step 1 Grievance for Defendant Sicotte for **NOT** treating the Plaintiff's chronic pain and this grievance was denied. **See Exhibit #4 - Document #1.**

12th of January 2013; The Plaintiff filed his Step 2 Grievance and this grievance was denied by the Health Service Administrator "gatekeeper", Defendant Blatnick. The Plaintiff states in this grievance that Defendant Sicotte is being untruthful about his ROM (range of movement) and his exercise capabilities which is apparent upon review of his medical records from the neurological specialist that treated him. The Plaintiff states that the Ibuprofen that Defendant Sicotte prescribed him is **NOT** helping him. The Plaintiff states that he is in pain and would like to be placed back on his neurotin. Defendant Blatnick as stated supra is the Health Service Administrator at the Limon Correctional Facility and she also has shown a deliberate indifference to the Plaintiff's medical needs, because if she would have properly researched into the Plaintiff's medical records Defendant Blatnick would have noticed that the medical provider that she is in charge of did in fact contradict her medical finds which was stated supra and Defendant Blatnick had the supervisory position and capacity to make things right and get the Plaintiff the proper medical treatment that he should have received and the pain proper pain medication. **See Exhibit #4 - Document #2.**

8th of February 2013; The Plaintiff filed his Step 3 Grievance and this grievance was denied, **Exhibit #4 - Document #3**. The grievance and the response to where the grievance was denied is **Exhibit #4 - Document #3A**. This document states that the Plaintiff has exhausted his administrative remedies in this matter.

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31st of January 2014; The Plaintiff on this said date declared a medical emergency for the severe pain that he was having in his left foot. A pain that the Plaintiff describes in **Exhibit #1 - Document #9** as being at that of 6 out of 10 on the pain scale. Defendant Blatnick listed on this same exhibit that she observed the Plaintiff walking with a limp. The Plaintiff denied any recent slips, trips or falls. The Plaintiff described his 2006 stabbing assault to Defendant Blatnick, telling her that he was stabbed in the back, that of the spinal cord area and that this injury left him with deficiencies to his left leg and foot. Defendant Blatnick upon visual exam listed on **Exhibit #1 - Document #9** that the Plaintiff had minimal swelling, noted to top aspect of foot. The Plaintiff informed Defendant Blatnick that the pain is worse at night and described the pain to be on the outer aspect of foot---underside to arch. The record states that Defendant Blatnick consulted with Defendant Massenburg, but Defendant Massenburg did **NOT** give the Plaintiff a physical exam. The Plaintiff feels that Defendant Massenburg (Medical Provider) should have done so considering the documented medical history that the Plaintiff has with his left leg and foot. (The record will reflect that on February 14th 2014 the x-rays revealed that the Plaintiff's left foot contained a 5th metatarsal fracture.) Defendant Massenburg as stated did **NOT** give the Plaintiff a physical exam, but rather ordered Defendant Blatnick to wrap the Plaintiff's foot in an ace wrap. The Plaintiff's left foot was treated as a sprain, strain or muscle ache. The Plaintiff was given 600mg of Motrin for 7 days and given ice for the swelling. The Plaintiff was ordered to continue to wrap his left foot in the ace wrap for seven (7) days. On this date **NO** x-rays were ordered to be taken of the Plaintiff's left foot. The Plaintiff was dismissed from medical to walk with **NO** assistance from the use of a pair of crutches and the Plaintiff was not given a second tier housing restriction. The Plaintiff's cell is on the second tier and these Defendant(s) took **NO** consideration that walking up stairs could further injure the Plaintiff's foot. **Exhibit #1 - Document #9 and #9B**, reflect that the Plaintiff's left foot pain was treated as a sprain, strain or muscle ache. The Plaintiff was **NOT** given an effective pain medication.

4th of February 2014; The Plaintiff was seen on this date for a follow-up for the pain that the Plaintiff was having within his left foot. On this date the Plaintiff was seen by one of the nurses within medical name Defendant Rosler. The Plaintiff informed Defendant Rosler that his left foot was still hurting and that his pain now is more severe and that it is at a 8 to 10 on the pain scale. Defendant Rosler noted within **Exhibit #1 - Document #10** that his pain is at a 8/10 and further notes that the Plaintiff is walking with a limp and is favoring the left. This is a clear indication that the Plaintiff was in pain and this is a documented observation by Defendant Rosler. Defendant Rosler consulted with Defendant Massenburg and once again Defendant Massenburg did **NOT** give this Plaintiff a physical exam. Defendant Massenburg ordered for the Plaintiff to receive eight (8) packets of Tylenol and was told to take this with his Motrin that has already been ordered. The Plaintiff was ordered to keep his foot wrapped in an ace wrap and elevated as much as possible. Defendant Massenburg ordered for x-rays to be taken when the x-ray technician was next on site at the Limon Correctional Facility. The Plaintiff once again was **NOT** provided with a pair of crutches to assist him in walking even when these Defendant(s) have observed the Plaintiff walking with a limp and favoring his left when walking. The Plaintiff was **NOT** provided with a second tier housing restriction. Even though it was obvious that the Plaintiff was in more pain than his prior visit on 01/31/2014. The Plaintiff was **NOT** provided with a pain medication that would relieve him of his pain.

5th of February 2014; The Plaintiff's left foot was finally x-rayed, but the results of this x-ray was **NOT** available on this date, because the x-rays had to be sent out of the facility to be read. **Exhibit #2 - Document #1** (These x-rays were **NOT** received back, the result of, till the 14th of February 2014. Took 6 days to get left foot x-rayed and then another 8 days to get the results. This a total of 14 days of walking on a fractured left foot and that of receiving **NO** proper medical treatment, deliberate indifference.)

C. NATURE OF THE CASE

11th of February 2014; The Plaintiff was seen by medical for a follow up for the pain that he has been enduring within his **left foot**. Defendant Rosler notes in **Exhibit #1 - Document #11** that the Plaintiff's x-ray results are **NOT** there yet. Defendant Rosler states within this exhibit that the Plaintiff's **left foot** remains slightly swollen. The Plaintiff informed Defendant Rosler that he feels "intense pain inside the foot", reports this pain to still be at 8 out 10 on the pain scale. Defendant Rosler notes that the Plaintiff's pain medication (Motrin and tylenol) has expired. Defendant Rosler once again consulted with Defendant Massenburg and once again Defendant Massenburg did **NOT** give the Plaintiff a physical exam, but rather gave a verbal order that the Plaintiff be given 2 Tylenol 3's. First dose in clinic and instructed to return the following day at am for the next dose. Defendant Rosler states that when the x-ray results come in the Plaintiff will have RTC. This statement made by Defendant Rosler is verification that the Plaintiff had **NO** specific treatment in place by these Defendant(s) for the pain that the Plaintiff was enduring within his **left foot**. Defendant Rosler states that the Plaintiff has a scheduled appointment to see Defendant Massenburg for the following Wednesday for **chronic nerve complaints**. The Plaintiff was dismissed from medical without a set of crutches to assist him in walking that would have relieved some of his pain. The Plaintiff also was **NOT** given a second tier restriction to keep him from having to walk up the stairs to his cell. This second tier restriction would have relieved some of the pain that the Plaintiff was having to endure and it would have lessened the possibility for any future injuries to occur. **Exhibit #1 - Document #11**.

12th of February 2014; The Plaintiff on this date saw Defendant Massenburg for the pain that he was having within his **left foot**. This medical visit was for the chronic pain in the Plaintiff's **left leg** due to the nerve damage that was a result from the 2006 stabbing assault mentioned supra. Defendant Massenburg states in **Exhibit #1 - Document #12** that the Plaintiff when he walks, the Plaintiff does so with some guarding when stepping down. This guarding is done, because the pressure applied to his **left foot** within a step causes the Plaintiff severe pain. Defendant Massenburg states in this mentioned exhibit that the Plaintiff foot turns medially when walking. Defendant Massenburg states that the Plaintiff's **left leg** has atrophy, no pain to palpation of calf muscle, decreased touch sensation lateral lower leg L5, muscle strength with extension 5/5. Defendant Massenburg proceeds to state that the Plaintiff's **left foot** shows mild inflammation, ROM reduced with flexion and extension, muscle strength against resistance, flexion/extension is 2/5, unable to fully flex/extend 5th, 4th and 3rd digits. Reduced Inversion/Eversion. Decreased touch sensation along S1 **left foot**. Foot Inverts with walking. **Exhibit #1 - Document #12** are the finding medical facts that Defendant Massenburg found when he finally conducted a physical exam on the Plaintiff. The Plaintiff states that Defendant massenburg should have conducted this physical exam prior to now, but instead showed a deliberate indifference to his medical needs. Defendant Massenburg submitted the physical therapy consult for the Plaintiff and for the Plaintiff to receive Neurotin for the pain that he was enduring from the nerve damage. Defendant Massenburg state that "Still awaiting x-ray results for **left foot**". As the medical document, **Exhibit #1 - Document #12** will reflect, the Plaintiff was **NOT** on any medication pain, because the Defendant Massenburg refused to give the Plaintiff a continuous and effective pain medication to relieve the pain. Defendant Massenburg subjected this Plaintiff to a wanton infliction of pain and suffering. The Plaintiff has made Defendant Massenburg aware of his pain and Defendant Massenburg has made a physical observation of the Plaintiff having difficulty when he walks, but Defendant Massenburg knowing of these facts and that the Plaintiff has recently been x-rayed to determine the problem in the Plaintiff's **left foot**. Defendant Massenburg dismissed this Plaintiff from the medical department without giving him a pair of crutches to assist him in walking and further did **NOT** consider giving the Plaintiff a second tier housing restriction to keep the Plaintiff from walking up the stairs which could cause further injury to his **left foot** and subject the Plaintiff to any more unnecessary pain.

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Exhibit #1 - Document #13 is the "Consultation Report" that Defendant Massenburg" submitted for the Plaintiff to receive physical therapy. This request was later denied on the grounds of the Plaintiff's injury being pre-existing.

14th of February 2014; The Plaintiff was seen by Defendant Massenburg for review of left foot/ankle x-ray. Defendant Massenburg states for x-ray findings;

"No evidence of acute ankle fracture, some elevation of the cortex and mild expansion of the mild diaphysis of the 5th metatarsal, "If" patient has hx (history) of 5th toe fracture, this may represent healing response."

Defendant Massenburg gives the Plaintiff a 60 day supply of Salsalate as a trial medication for pain control. Defendant Massenburg proceeds to discuss the unpredictable nature of healing process and future pain/inflammation. The Plaintiff would like to state here for the record that Defendant Massenburg states;

"I/M states it could have been when his foot was ran over by a car".

The Plaintiff states that he has **NEVER** told Defendant Massenburg that his foot was ran over by a car. The Plaintiff would like this Honorable Court to review **Exhibit #2 - Document #1** which states that the Plaintiff's left foot has a healing fracture at the 5th metatarsal. The Plaintiff states that he has never had a 5th metatarsal fracture or such in his left foot. The Plaintiff was dismissed from his medical visit, the Plaintiff was **NOT** given a pair of crutches to assist him in walking and to assist with the weight that is applied to the foot when walking which subjects this Plaintiff to much pain.

4th of March 2014; The Plaintiff declared a medical emergency for left foot pain. The Plaintiff states that when he stood up and stepped down and proceeded to stand up. The Plaintiff states that he heard a pop/crack, sound of a bone breaking. The Plaintiff then felt severe pain to his left foot. The Plaintiff states in this **Exhibit #1 - Document #15** that the Salsalate is **NOT** giving him any relief from the pain that he is feeling within his left foot. On this date the Plaintiff was seen by Defendant Massenburg. The Plaintiff would like to point something out for this Honorable Court within this exhibit. Defendant Massenburg proceeds to state:

"Today, he states could have been a car running over his foot or when clowning around on the streets".

Defendant Massenburg said previously, supra, that the Plaintiff said that statement on 02/14/2014, but states that he said it on this date. Defendant Massenburg proceeds to fabricate more to this alleged statement by further stating "or when he clowning around on the street". Again, the Plaintiff states for this complaint that he **NEVER** made such a statement to Defendant Massenburg. Defendant Massenburg proceeds to state;

"He was observed moving the foot in different positions (extending toes, inverting foot, dorsiflexion, essentially the same as on previous exam".

To the Plaintiff it sounds as if Defendant Massenburg is contradicting his physical medical exam that he performed on the Plaintiff on 02/14/2014, **Exhibit #1 - Document #12**. The Plaintiff would like to further state that he **NEVER** told Defendant Massenburg that he "could easily just "shoot up" back in his dorm, **Exhibit #1 - Document #15**. The Plaintiff would like to inform this Court that if the Plaintiff would have said such a statement then the facility security would have gotten involved and to further inform this court. There was correctional facility security standing present with the Plaintiff and Defendant Massenburg during this medical emergency visit to medical.

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Defendant Massenburg goes on to state;

"He was offered crutch but refused and per nurse conversation stated he didnt want to be tempted to use them in the wrong way. On that statement, crutch was denied when he returned to clinic requesting the crutch. He was offered ice, but refused that as well. HeHe states that Naproxen is not going to help but chose to take the medication. He was provided initial dose Naproxen 500mg x1 today. He was on Salsalate and provided various excuses about why he was not consistently taking the medication".

The Plaintiff did **NOT** state that he did not want to be influenced to use the crutch in the wrong way. This was fabricated by one of the nurses that the Plaintiff has been having problems with which still continues to todote, nurse Defendant Rosler. Defendant Rosler even went as far as calling the shift commander and stated that the Plaintiff was shooting up drugs back in his housing unit. This turned out to be false! The Plaintiff had to submit a drug test and then security came to the Plaintiff's cell and conducted a cell search which turned up nothing. **The Plaintiff would like to state for this Honorable Court that the drug test that the Plaintiff's drug test came back NEGATIVE, No drugs of any kind in his system!** The Plaintiff wrote a grievance on Defendant Rosler for making a false statement to security that he was shooting up drugs. **See Exhibit #7 - Document #1.**

The Plaintiff was **NOT** given an effective pain medication when he advised the Defendant(s) that Salsalate and Naproxin was **NOT** providing him with pain relief. Defendant Massenburg did order for the Plaintiff's left foot to be x-rayed when the x-ray technician was on site at the Limon Correctional Facility.

5th of March 2014; The Plaintiff's left foot was x-rayed again on this date. The results of this x-ray was **NOT** made available again the same day, because these x-rays had to be sent out of the facility to be read. **(The results of this x-ray were made available to the medical provider (Defendant Massenburg) on 03/11/2014, this again is 6 days after the x-rays were taken.)**

Let **Exhibit #2 - Document #3** reflect that the Plaintiff's left foot is **NOW** containing a fracture in the 2nd metatarsal that is consistent with an acute fracture. Further stating there is a healing fracture through the proximal diaphysis of the 4th metatarsal. An old fracture, which appears healed through the mid diaphysis of the 5th metatarsal.

11th of March 2014; The Plaintiff on this date saw Defendant Massenburg for the pain and fractures that are in his left foot. Defendant Massenburg states within **Exhibit #1 - Document #16**, Plaintiff ambulates with no assistance, **slight limp left leg**, states that yesterday the foot did not hurt but today it is hurting. Objective: **Left foot x-ray results.** Acute fracture of the 2nd metatarsal. Sub acute-chronic fracture of proximal 4th metatarsal and a chronic fracture of the 5th metatarsal. Alignment anatomic. Passive ROM of Metatarsals WNL, active plantar flexion limited **due to pain complaint**, very mild inflammation, pedal pulse WNL. Advised to wear Post-Op Shoe x 3 weeks with follow up x-ray. Ortho consult requested. X-ray reports included. The Plaintiff states that **NONE** of the Plaintiff's x-ray reports reflect that his fractures are chronic in nature. Defendant Massenburg implements "chronic" into his medical reports to try and cover the deliberate indifference for the Plaintiff's medical needs that he has shown this Plaintiff in the earlier stages of the Plaintiff's complaints about his left foot problems and pain that he was recieving.

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Exhibit #1 - Document(s) #17 and #18 are the documents that Defendant Massenburg submitted for this Plaintiff to go and see a Orthopedic at Denver Health. The Plaintiff asserts that Defendant Massenburg and Defendant Blatnick (Health Service Administrator) should have sought this action for the Plaintiff to see a Orthopedic when a contiuation of fractures were appearing within the Plaintiff's **left foot** and when these fractures were unexplainable in nature.

14th of March 2014; The Plaintiff submitted a Informal Grievance, stating;

"On 3-11-14 I went to medical and my x-ray results were in. And two additional fractures were found, although the two x-ray reports some what contradict each other, probably oppinions of two different people reading the x-rays. The provider gave me a boot, I don't know why this wasnt done the first time in order to prevent further injury. I told the PA that I lived in 2B-2-14 and wanted to move to 2B-1-1 so I didnt have to walk upstairs all day. He said I dont meet the criteria to move to that cell. I told him Im still in pain and he is aware of condition regarding my nerve damage in my **left leg** and the **pain** in my foot shoots pain up my leg and it **causes more frequent and severe muscle spasms that are very painful** and I have to use physical force and body weight to straighten leg and calf muscle but cant put weight on foot. I asked for something to help with pain and spasms. I started with Salsalate and then went to Naproxen and now on Tylenol. I tell him that I'm in **pain** and then given a weaker medication each time that dont work. As a remedy I want an effective and consistent medication, **not 2 week prescriptions**".

The Plaintiff asks within this grievance to be placed on the first tier so he would **NOT** have to walk up the stairs, but the provider, (Defendant Massenburg), denied him the ability to be placed on the first tier so he would not have to walk up the stairs. Actually moreless stating that a fractured foot does not meet the criteria. The Plaintiff he states that he is pain and wants a continuous and effective medication. The Plaintiff states this, because Defendant Massenburg would **NOT** prescribe him a continuous and effective pain medication. Defendant Massenburg would make the prescriptions for 3 days, 7 days, 14 days, and these would run out and the Plaintiff would have to wait days and some times up to a week in pain to get the prescription rewritten. The Plaintiff states that this grievance was actually moreless denied, because the Plaintiff's remedy was **NOT** met. This grievance is **Exhibit #6 - Document #1**.

14th of March 2014: The Plaintiff submit's another Informal Grievance on Defendant Rosler;

"On 3-11-2014 I was in medical for falling and further injuring myself. I was frustrated with the PA for deliberately ignoring my plea for something with the pain. I told PA and nurse Leon that I wasnt trying to catch a buzz or fake an injury to get high on pain meds. I was waiting for nurse Leon to get crutches and Tina Rosler (Defendant Rosler) had arrived in medical. She could easily see that I was in pain and frustrated when Leon gave me crutches and I asked if I could please get something to get my foot to stop hurting. CO's Wright and Kieble were present when this all took place. On my way out of medical Tina Rosler said, "leave those here if you're gonna do something you're not suppose to do....Oh, and have a nice day sir". She said this rudely with sarcasm and the odvious attempt to cause further frustration and she had a smile while her and another nurse laughed at me! In pain and frustration I said "Youre right, I wouldnt want to be tempted, I'll come back after lunch. Tina (Defendant Rosler) called the unit and reported threats and said that I demanded pills or would go get high. Tina (Defendant Rosler) wasnt even on shift when I spoke with PA (Defendant Massenburg) and nurse Leon so she is a liar. CO's present even said I never made those comments. As a remedy I want Tina R. (Defendant Rosler) reprimanded for falsely reporting serious allegations that were not found to be true, she violated her code of conduct. I also want to be treated with respect and in a professional manner".

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The Plaintiff's grievance was denied! The Plaintiff stated supra that his cell was searched and that he had to take a drug test due to a nurse making a false report. This is that what the Plaintiff stated supra. The nurses such as Defendant Rosler go beyond their professional duty to torment the Plaintiff and subject him disciplinary sanctions. If the Plaintiff would have been shooting up some sort of drug then the Plaintiff's drug test would have provided that proof, but that was not the case, because the Plaintiff's drug test came back negative. CO's Wright and Kiebel even affirmed that the Plaintiff never made such comments. This grievance is listed as **Exhibit #7 - Document #1**.

18th of March 2014; The Plaintiff on this date had a meeting with Defendant Blatnick and the purpose of this meeting was to discuss how dis-satisfied the Plaintiff was with the medical treatment that he has been receiving. This meeting that the Plaintiff had with Defendant Blatnick lasted approximately for 1 hour and 15 minutes and there was many things discussed within this meeting. **Exhibit #5 - Document #1**. The Plaintiff started off by asking why he was **NOT** receiving follow-ups and the Defendant Blatnick stated that those were done according to the provider (Defendant Massenburg) and **NOT** the inmates. The Plaintiff wanted to know why nothing was being done for his **left foot and the pain** that he was enduring. The Plaintiff informed Defendant Blatnick that medical has been aware of his **left foot** injury since 01-31-2014 and it is now 03-18-2014 and nobody is taking this matter with his **left foot** seriously. Defendant Blatnick said it could take up to 8 weeks for his "sprain" to heal and even longer, because the Plaintiff is constantly walking on it. Let the Plaintiff remind this Honorable Court that these Defendant(s) gave this Plaintiff **NO** crutches to assist him in walking. Defendant Blatnick stated that X-rays only show bone and not sprains. The Plaintiff started producing his CDC medical records and when he did Defendant Blatnick told the Plaintiff if he was going to shut her down then the meeting was over. Defendant Blatnick told the Plaintiff that nothing in his medical file points to a broken or fractured **left foot**. The Plaintiff had his x-ray report with him and he produced that for Defendant Blatnick to review. Even when Defendant Blatnick saw that the x-ray report stated a fracture, Defendant Blatnick stuck with the fact that his foot was sprain. This is because Defendant Blatnick on 01/31/2014 treated the Plaintiff's **left foot** injury as a sprain, strain or muscle ache. **See Exhibit #1 - Document #9**. He went on to state that he also has a second x-ray report that reflects that he has additional fractures in his **left foot** since the first x-rays were taken. The Plaintiff went on to ask why Defendant Massenburg told the Plaintiff that he would have a KOP med-card for Salsalate and that he could pick it up at med-line so he could take it when he needed, but when he went to pick up the med-card he was instructed that he would have to pick up the Salsalate at med-line every time he wanted to take it. He was also further instructed by the med-line that his Salsalate is **NOW** only prescribed for every 14 days and for no-longer. The Plaintiff had talked to Defendant Rosler and when he asked what was he to do when the 2 week (14 day) prescription ran out. Defendant Rosler instructed him that he would have to fill out a medical request form, be scheduled an appointment, seen again by the provider, and then the prescription would be re-written for Salsalate. The Plaintiff also informed Defendant Blatnick that Defendant Rosler also told the Plaintiff that he could purchase pain medication off of canteen in between prescriptions. The Plaintiff asked Defendant Blatnick why was he being given only two week prescriptions. He further wanted to know why he was being down graded in the pain medications that he was taking. Telling the Defendant Blatnick that he was first on Salsalate, then reduced to Naproxen, and then reduced to Tylenol. Defendant Blatnick told the Plaintiff that the providers are taking a **"conservative turn toward self medications"**. The Plaintiff asked Defendant Blatnick what that meant. Defendant Blatnick then told the Plaintiff that Naproxen, Motrin, Ibuprofen, Tylenol, and Salsalate cause bleeding and ulcers in the stomach and can ruin your liver. The Plaintiff asked Defendant Blatnick who was making these decisions to hand out 2 week

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prescriptions and Defendant Blatnick said "the providers here at LCF". The Plaintiff told Defendant Blatnick that this is **NOT** what was televised on the prison news channel 6. That according to the LCF prison news channel 6 it says that the Chief Medical Doctor has decided to no longer do Salsalte as a KOP medication. Defendant Blatnick then stated that the Chief Medical Doctor and the Supervisor over the pharmacy decided to take certain self medications and make them available through the med-line. The Plaintiff asked why they decided to do that. Defendant Blatnick went on to explain that if they gave 100 inmates a 30 day supply card of Naproxen that that is a lot of meds and by having them come to the med-line to get them they would only come to get them if needed. The Plaintiff then said he was confused and told Defendant Blatnick that he did not get why they were only doing two week prescriptions, that an inmate could still come to the med-line 30 days a month and the same amount of meds were still being used. The Plaintiff went on to state that he does not get why the Chief Medical Doctor and Pharmacy Supervisor are only letting inmates get two weeks of medication for pain. **Defendant Blatnick said that CDC is given a budget and the cost of their medications they purchase are going up by the pharmacy, Defendant Blatnick went on to say, "its more cost effective".** She said it has to do with budget cut, Defendant Blatnick. The Plaintiff went on to address how the nurse were not very polite and were rude to him and even went as far as making false allegations. The Plaintiff even informed Defendant Blatnick how Defendant Rosler called the housing unit and said that she witnessed the Plaintiff saying "if you don't give me pain meds I'm going to go to the unit and get high then". The Plaintiff told her (Defendant Blatnick) how Defendant Rosler tried to have him put in segregation demanding that an officer write up a report. The Plaintiff asked who it was who called the unit and made that allegation, but Defendant Blatnick would not tell so the Plaintiff informed him that someone on security staff already told him that it was Defendant Rosler who made that call. The Plaintiff wanted to know why she (Defendant Blatnick) allowed her staff (Defendant Rosler) to make such false allegations and nothing happens to that member of staff when the allegation was proven to be false. **(Defendant Blatnick is the Health Service Administrator).** The Plaintiff informed Defendant Blatnick that Defendant Rosler violated the Colorado Department of Corrections Administrative Regulation for Code of Conduct - #1450-01. Defendant Blatnick told the Plaintiff that she would not give the Plaintiff the name of the nurse anyway so could use it in a grievance against that nurse, (Defendant Rosler). The Plaintiff told her again he just wanted to see if Defendant Blatnick would do what is right by holding her staff accountable. The Plaintiff also pointed out how one day when he went to the med-line and forgot his identification card and nurse Ashley made him go all the way back to get it and then he observed Nurse Ashley excusing another inmate who did not have his, but instead of Nurse Ashley making that inmate go get his identification. Nurse Ashley just looked at the inmates tag on his shirt and entered in his number and gave him his medications. The Plaintiff stated how he brought that observation up to Nurse Ashley when he came up to the med-line and Nurse Ashley said the Administrative Regulation has just been changed. The Plaintiff stated that this was just harassment! The Plaintiff even brought up how Defendant Sicotte stated that he just wanted pain medication so he could get high and how Defendant Sicotte would not give him treatment for his nerve damage in his left leg. **Defendant Blatnick said that is why Defendant Sicotte was "FIRED"!** Defendant Blatnick told the Plaintiff that Defendant Sicotte deliberately denied inmates medical treatment, Defendant Blatnick made it a point to say she is gone for that reason. The Plaintiff then pointed out that Defendant Blatnick herself denied the Plaintiff's Step 2 Grievance, **Exhibit #4 - Document #2**, a grievance that the Plaintiff filed on Defendant Sicotte for not providing him with proper medical treatment. The Plaintiff stated that she, defendant Blatnick, should have looked into his medical records herself since she was answer that Step 2 Grievance and she would have found/seen that the Plaintiff needed treatment for the nerve damage in his back and in his left leg. Defendant Blatnick told the Plaintiff that she can **NOT** make a provider do anything or go against what they want to do as far as treatment goes. The Plaintiff informed Defendant Blatnick that nothing is being done for the pain. The Plaintiff went on to explain how he can't sleep at night and

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would like something for the pain so he could get some sleep. Defendant Blatnick stated that she would get on top of it and focus on trying to get him physical therapy and something for the pain. Defendant Blatnick said she would also try to get the Plaintiff more than a two prescription for the pain. The Plaintiff mentioned how she just said that she could not go against a providers orders. Defendant Blatnick said she would talk to her provider (Defendant Massenburg) and her nurses to try and get something done. Defendant Blatnick also mentioned that she was going to try and see about getting the Plaintiff's **left foot** put in a cast. The Plaintiff thought to himself how not an hour ago Defendant Blatnick said that his **left foot** injury was that of a sprain, but now she mentioned having his **left foot** put in a cast. Defendant Blatnick said she would see the Plaintiff on the 20th of March and that she would try to have some answers for him. **See Exhibit #5 - Document #1.**

Exhibit #1 - Document #19, this Defendant Blatnick's entry into the Plaintiff's medical records on this mentioned meeting with the Plaintiff.

21st of March 2014; The Plaintiff wrote a letter and sent it to Defendant Blatnick. The Plaintiff informs Defendant Blatnick that he is in pain, stating a 8th Amendment violation. Pointing out the fact that Defendant Blatnick is aware that the Plaintiff's **left foot** contains 3 fractures and how nothing is being done. The Plaintiff tells Defendant Blatnick how when he walks and applies pressure to this **left foot** he recieves severe pain. This Plaintiff has not been given crutches to assist him in walking! The Plaintiff states how he is having muscle spasms and how they are more frequent and intense with pain. The Plaintiff goes on to ask Defendant Blatnick for some sort of medication for these muscle spasms. The Plaintiff points out that Defendant Blatnick is in charge of medical and is requesting her intervension into this matter.

21st of March 2014; The Plaintiff files a Step 1 Grievance on Defendant Massenburg stating;

"My x-rays show that I have a **NEW** fracture in my 2nd and 4th metatarsal and these fractures were **NOT** present in the comparison x-ray of 02-05-2014. This 02-05-2014 x-ray shows a 5th metatarsal injury that is **NOT** present in my medical injury history. When this 5th metatarsal injury became present in my 02-05-2014 x-ray Massenburg should have taken precaution and stabilized the injury in a cast or proper fixation, but yet did nothing and the result of that was a **NEW** fracture in my 2nd and 4th metatarsals. After subjecting me to this **NEW** fracture, **more pain and suffering**, (8th Amendment Violation), Massenburg provides me with a temp. foot boot, **No** work restriction, **NO** 1st tier housing restriction, and **NO** continuous and effective pain medications. I have an expired prescription for Salsalate and Tylenol. I have permanent nerve damage resulting in chronic pain. As noted in medical history I do get intense muscle spasms and the recent problems with my foot make matters worse. My pleas for melp have been ignored and kites are **NOT** being recieved. **Nurses telling me to puchase pain meds off of canteen.** I am constantly in pain and am **NOT** being taken seriously".

This grievance was answered by Defendant Massenburg and Defendant Massenburg denied the Plaintiff's grievance. Defendant Massenburg goes to put the blame on some trauma-fracture to the Plaintiff's left foot while on the outside of DOC. The Plaintiff has stated supra that he has **NEVER had such an injury to his left foot prior coming to CDC.** The Plaintiff has already stated supra that he did not deny the use of the crutches and this attempt is to blame the Plaintiff for the consistant injuries to his foot.

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21st of March 2014; The Plaintiff on this date filed his Step 1 Grievance on Defendant Rosler and this grievance was denied with the explanation if he was unsatisfied with this response that he could proceed to the next step within the grievance process.

24th of March 2014; The Plaintiff was brought to the medical department to meet with the Defendant Blatnick to be informed of the findings in regards to outside consults and pain management. The Plaintiff was told that his physical therapy was denied on grounds that it was a "pre-existing condition". The Plaintiff explained that he was still in pain and would like a continuous and effective pain medication and the Plaintiff stated that he felt that this was deliberate on the part of medical. The Plaintiff described to Defendant Blatnick that his pain has increased- "shooting electrical pain from his foot all the way to his gluteal muscles and said he would like to find a solution to dealing with this pain. Defendant Blatnick said that the Plaintiff can pursue an outside opinion based on AR 700-21. The Plaintiff did state that he would be seeking legal actions. Defendant Blatnick said that the pain management would be discussed with Defendant Massenburg. **See Exhibit #1 - Document #21.**

Exhibit #5 - Document #2, the Plaintiff's affidavit on the Meeting with Defendant Blatnick.

The Plaintiff informed Defendant Blatnick that Defendant Massenburg informed him that the physical therapy was recommended for his left foot and she (Defendant Blatnick) told him that since his foot was a result from nerve damage and **that medical thinks it was a pre-existing fracture they do not have to do physical therapy.** Defendant Blatnick told the Plaintiff that the insurance company looks at his mobility, if he can get to and from chow. Then he does **NOT** need physical therapy! They do **NOT** look at how well your mobility is, just that you get there, not how well you get there. The Plaintiff went ahead and addressed the fact that nothing is being done about his pain treatment. Defendant Blatnick told the Plaintiff that CDC has been **NOT** wanting to treat pain with medication because they want to be focused on the result in the long run. The Plaintiff told Defendant Blatnick since physical therapy is being denied so what other way is readily available that the medical industry has available for treating his pain? Defendant Blatnick told the Plaintiff that they have given him every medication available (i.e. in the formulary) that they have on hand. The Plaintiff expressed that the Tylenol 3 that was given to him was only for two or three days then it was back to being in pain. Defendant Blatnick asked the Plaintiff what is opinion was as to what kind of medication for pain would work and he said that he felt his honest opinion would viewed as if he was kind of junky seeking to get high since that is how he has already been treated by Nurse Rosler, (Defendant Rosler). The Plaintiff said that even Defendant Sicotte stated that he was only seeking pain medication so he could get high. The Plaintiff stated that it appears that the Limon Correctional Medical Department looks at all inmates as if they just want to get high and does **NOT** care if the inmate is in pain. The Plaintiff stated that he felt that Tylenol 3 for pain and a muscle relaxer such as Baclofen for the muscle spasms would work in the am and in the pm. He stated that maybe Neurotin for the nerve damage with Baclofen might work, but it needed to be continuous and not two or three dose stuff that Defendant Massenburg likes to prescribe. The Plaintiff told Defendant Blatnick that it has come to the point that he knows **NOT** to expect anything from medical, because medical will not do anything for him. Instead they (medical department) would rather show a deliberate indifference to his medical needs and would rather subject to to a unnecessary wanton amount of pain and suffering all for the sake of the budget. The Plaintiff informed Defendant Blatnick how the nurses at med-line tell him that he can just purchase medication for his pain from the canteen. The Plaintiff told Defendant Blatnick that maybe he should take it to the next level and into the courts.

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Defendant Blatnick told him that is what he might need to do to get the results that he wanted. Defendant Blatnick told him that also might need to be done to get changes made. The Plaintiff told Defendant Blatnick that Defendant Massenburg has done nothing but show a deliberate indifference to his medical needs.

25th of March 2014; The Plaintiff was seen by Defendant Massenburg and as continued to state within the Plaintiff's medical records that "he has history of unknown etiology of left foot pain possible from a car running over his foot". Defendant Massenburg states that new x-ray revealed 2nd metatarsal fracture, goes on to state that pain and the Defendants **left leg** locking up in bent position and keeps him up at night due to pain. State that when the Plaintiff's **left leg** locks up, he has to pull it straight. Plaintiff states that the Tylenol 3 did take away some of the burden of the pain and states that the Gabapentin (Neurotin) is not helping him. Plaintiff states this, because Defendant Massenburg will not prescribe the Neurotin prescription in a continuous fashion so the medication can become effective. Plaintiff stated that he use to do light self weight squats to get muscle movement within his leg and this did help him. Defendant Massenburg notes that the Plaintiff's **left foot** has some mild swelling dorsal aspect of the foot, ROM is good but **NOT** full, unable to flex toes without pain complaint, passive ROM good, tenderness with palpation of proximal metatarsal region. Defendant Massenburg noted Ortho consult pending and Left x-ray scheduled. Advised the Plaintiff periodic time without ace-wrap on foot/leg specifically when resting and elevating leg. The Plaintiff is to kite medical on effectiveness of new pain medication. Defendant Massenburg does a discontinuation on the Neurotin. **Exhibit #1 - Document #21.**

25th of March 2014; Defendant Massenburg did a medication change. **Exhibit #1 - Document #22.**

26th of March 2014; A provider, Kucher, ordered the Plaintiff a 14 day supply of Tylenol 3 due to the pain that the Plaintiff was havilng within his **left foot**. The Plaintiff was **NOT** examined by Kucher, but she placed the prescription for him. **The Plaintiff states why couldnt Defendant Massenburg have placed such a lengthy prescption?**

2nd of April 2014; the Plaintiff's **left foot** was x-rayed. The results state that the Plaintiff's **left foot** **NOW** reflects that the **left foot** has various ages of healing involving the 2nd, 4th, 5th metatarsal fractures with persistent fracture lines identified at the 2nd and 4th metatarsal fractures. **Exhibit #2 - Document #4 and #5.**

3rd of April 2014; The Plaintiff filed his Step 2 Grievance on **NOT** being provided with reasonable adequate medical care. This grievance was denied by Defendant Blatnick. **Exhibit #6 - Document #3.** Defendant Blatnick states that he has been seen six (6) times between the medical provider and nursing staff. The Plaintiff states that the "graveman" of his medical problem with his **left foot** is **NOT** being met.

10th of April 2014; The Plaintiff on this date filed his Step 2 Grievance on Defendant Rosler for Code of Conduct, due to the false report this Defendant Rosler made stating that the Plaintiff was using illegal drugs which Defendant Rosler's false report was proven to be false. The Plaintiff stated in this grievance that he has had to take three (3) drug test in six (6) weeks and has had to endure four (4) cell searches. The drug tests and cell searches all turned up **NO** findings of drug use and if the Plaintiff was using drugs then the Plaintiff's drug test would have turned up positive. The Plaintiff's request was that Defendant Rosler be properly disciplined and the Plaintiff states that ~~maybe~~ he should include this as a retaliatory claim against medical, these actions by Defendant Rosler being

C. NATURE OF THE CASE

that of harrassment. Let **Exhibit #7 - Document #3** reflect that this Grievance was answered by Defendant Blatnick, the Health Service Administrator, the "gatekeeper", and she denied the Plaintiff's grievance. This is proof that Defendant Blatnick is in agreement with the actions of harrassment and false repoting by her staff that the Plaintiff was using illegal drugs when the Plaintiff's drug test's and cell searches have found **NO** evidence of drug use.

14th of April 2014; The Plaintiff on this said date declared another medical emergency due to the severe pain that the Plaintiff was having within his **left foot**. The Plaintiff on this date was seen by Defendant Dang. On this date the Plaintiff reported that this morning when he went to stand up, his foot bent in and he recieved pain at left foot arch. The Plaintiff states that the pain was sharp and throbbing; reporting the pain to be at 7 out of 10. The Defendant Dang makes some false idiotic reports within the Plaintiff's medical file, stating;

"When I concerned if offender had any liver issues, he stated with me and officers that, "No. No Hep C or B, but I wish I had. It is cool". I told him, "Believe me. It is not as you thought. You won't want to get it."

The Plaintiff states that he never made such a statement as wanting Hepatitis period and No individual in their right mind would. This is another attempt by medical staff that is under the supervision of Defendant Blatnick to make this Plaintiff seem crazy or as if an attempt to justify the deliberate indifference to the Plaintiff's medical needs by these named Defendant(s).

The Plaintiff asked to have his **left foot** x-rayed, but that was denied stating that the Plaintiff had an up coming medical appointment with a foot specialist. Defendant Dang gave the Plaintiff six (6) packegaes of Tylenol and instructed him to take one package at a time, three (3) times a day. Defendant Dang goes to state that the Plaintiff staes that these six (6) packages of Tylenol are only enough for that evening. The Plaintiff again never made such a statement and knows that taking that many Tylenol can cause harm to a person's stomach. The Plaintiff states that this is another attempt to make him seem as if his only goal was to seek some sort of pain medication.

The Plaintiff would like to assert that Defendant Dang makes another false report with-in the Plaintiff's medical records stating;

"When I was doing in the evening med-line,
I saw offender Rabidue walked by outside the medline. He walked fine; no accute distress noted."

The Plaintiff would like to point out how fabricated this statement is. The Plaintiff had **NO** medication present to take at med-line and this is a fact that can be found on **Exhibit #1- Document #24** that reflects this Plaintiff had no medication to pick up at med-line. The Psyllium Fiber that the Plaintiff takes is a medication that this Plaintiff keeps within his cell and further more if you go to the med-line and you do not have medications to take, Then security will write you a disciplinary incident report and you recieve a sanction. The Plaintiff was excused from medical without crutches to assist him in walking or any pain medication to relieve him of his pain within his **left foot**. See **Exhibit #1 - Document #24 and #25**.

C. NATURE OF THE CASE

14th of April 2014; The Plaintiff has done a Affidavit, **Exhibit #5 - Document #3**, where he states how he has endured another injury to his **left foot** on this said date. Stating that in an attempt to stand up he accidentally put weight on the front part of his **left foot** and felt a popping or a crunching sound. The sound to that of a bone breaking. When this injury happened the Plaintiff went to the unit 2 security office and spoke with Correctional Officer Salters and this officer called medical to ~~enquire~~ if they could do anything for the Plaintiff, but medical said that he had to do a medical emergency. Officer Salters told him that she spoke with Defendant Rosler and this Defendant Rosler told Officer Salters that there was nothing they could do for the Plaintiff, further stating that the Plaintiff has been approved to see a foot specialist and will **NOT** recieve any treatment from the Limon Correctional Facility - Medical Department until he is seen my the foot specialist. The Plaintiff states that he wants to do a medical emergency and goes on to state that he feels unsafe in LCF, because of Defendant Rosler on reason of her past allegations on the Plaintiff. The Plaintiff decided to wait until the next shift came on to do a medical emergency. The Plaintiff was escorted to medical by security officers Martinez and Lacik. Before leaving unit 2, the Plaintiff was placed in ankle restraints and hand-cuffs. The Plaintiff states that he has **NEVER** prior to this has he ever been placed in ankle restraints. The Plaintiff states it took him a long time to walk to medical because it was hard for him to walk in ankle restraints. **(Let the Plaintiff remind this Honorable Court, this Plaintiff's previous x-rays has declared this Plaintiff to have numerous fractures in his left foot.)** The Plaintiff's asserts that he was in severe pain by having to walk in these ankle restraints with his left foot already being fractured and this Plaintiff was having to walk on his own without the assistance of crutches or a cane. When the Plaintiff arrived in medical, the hand-cuffs where taken off the Plaintiff, but the ankle restraints was left on him. Defendant Dang told the Plaintiff to take off his shoe and the post op shoe that he had on along with his socks. The ankle restraints limited his ability to take off these items and brought this Plaintiff under pain, because he had to try to bring his legs and feet up to remove these items. The Plaintiff even had to get on the gurney by himself, no assistance from security or the Defendant Dang. These individuals just stood and watched this Plaintiff struggle to remove the items requested by this Defendant Dang. This even continued even after the Plaintiff asked for some help and recieved none, making his foot hurt more!

See the previous date of 14th of April 2014 and see Exhibit #1 - Documents #24 and #25.

The Plaintiff asks this court to review Exhibit #1 - Document #24 and how this Defendant Dang has a language barrier and writing barrier which is and has affected the Plaintiff's medical record documentation and communication with medical. The Plaintiff states supra that Defendant Dang make statements that Hep being cool, but in fact the barrier of communication is on that of the Defendant Dang.

After Defendant Dang finished examing this Plaintiff's **left foot** and his right foot, the Defendant Dang being done with the physical exam part. The ankle restraints were finally removed so the Plaintiff could put his socks, his shoe and post op shoe back on. The Plaintiff states, why wasn't this removed when I came into medical, but the Plaintiff did not recieve a response. The Plaintiff asked for some crutches, pain relief, and this Defendant Dang became angry and said;

**"NO Tray, NO Provider, NO Meds, NO Ice, and
definitely NO Crutches!"**

Standing at the end of the hallway, the Plaintiff could see Defendant Blatnick, the Limon Correctional Facility Health Service Administrator, in her office so the Plaintiff asked if he could talk to Defendant Blatnick. Defendant Dang said that;

C. NATURE OF THE CASE

"Nicole (Defendant Blatnick) is not here today!" The Plaintiff said that he must be looking at her twin sister then!

The Plaintiff was excused from medical without the exam from the medical provider and the Plaintiff was not provided with a pair of crutches, nor was he given a second tier housing restriction, and nor was the Plaintiff given a continuous and effective pain medication.

Affidavit is Exhibit #5 - Document #3.

17th of April 2014; The Plaintiff on this date was transported to the Denver Health Center in Denver Colorado to see a ortho-pedic foot specialist and this doctor conducted a physical exam and performed several x-rays on the Plaintiff. **Exhibit #8 - Document #2** states;

"Taken today at this visit and it should be noted that there are transverse fractures just proximal to the head of the second and third metatarsal with bony callus formation in the second metatarsal fracture. There is also some evidence of healing with bone callus noted in the shaft of the fifth metatarsal, as well as the fourth metatarsal. There is some slight plantor flexion of the heads of the second and third metatarsals, but overall, the alignment is good."

ASSESSMENT: Fractures of metatarsals 2 through 5 left foot with healing of the fractures of the fourth and fifth metatarsals and continued healing of the second and third metatarsals.

The orthopedic foot doctor stabilized the Plaintiff's left foot in a Cam walker boot. The doctor states;

"I feel that if we immobile the patient's ankle to reduce the tendon motion over the metatarsals through the toes and place him in a rocker bottom type of foot gear, then he will not be trying to bend his toes at the metatarsophalangeal joints and this will likely allow him to heal much more effectively. He seems to have healed the fractures in the fourth and fifth metatarsals just wearing a postop shoe, so therefore, I do feel that he will heal up more effectively with no continued pathology in the second and third metatarsals. He is to follow up with me here in the CCMF clinic in six weeks."

See Exhibit #8 - Document #1, Exhibit #8 - Document #2, Exhibit #8 - Document #3, and Exhibit #8 - Document #4.

Let the record reflect that the Plaintiff requested to speak to Defendant Blatnick, the Health Service Administrator that has authority to see that the Plaintiff receive adequate medical care and a physical exam by Defendant Massenburg, the medical provider, this request was denied. Denying the Plaintiff a physical exam that could have given him adequate medical care and the possibility from receiving more injuries.

18th of April 2014; The Plaintiff's mother, Raeleen L. Woodbury, was concerned about the Plaintiff her son, because she has spoken several times on the phone and the Plaintiff informed his mother that his left foot contains several fractures and these named Defendant(s) were not providing him with adequate medical care. Raeleen Woodbury called the Limon Correctional Facility requesting to speak to Defendant Falk, the warden for the Limon Correctional Facility. Raeleen Woodbury outlines her conversation with Defendant(s) Falk, Blatnick, and a person whom she is understanding to be the ADA Co-ordinator by the name of Dave. The Plaintiff was told by Defendant Blatnick that this ADA individual was Defendant Long, a major within Administrations at the Limon Correctional Facility.

C. NATURE OF THE CASE

This phone conversation that Raeleen Woodbury had with these named defendant(s) was conducted over a speaker phone and in a conference call fashion. These named Defendant(s) made it a point to state that the Plaintiff has been seen Six (6) times so far by the medical department, practically stating atleast he is being seen and this factor is stated in the responses in the grievances that the Plaintiff has filed. Seems these named Defendant(s) seek to justify their deliberate indifference actions to the Plaintiff's medical needs by just the fact they are seeing him within the medical department. Raeleen Woodbury informed these named Defendant(s) that although the Plaintiff was seen six (6) times does **NOT** constitute treatment or receiving adequate medical care. The Defendant(s) tell Raeleen Woodbury that the Plaintiff refused a medical boot for his left foot back in March, but let the medical records reflect that **NO WHERE** within the Plaintiff's medical records is there a recording of such a thing as to the Plaintiff refusing a boot for his left foot back in March. These named Defendant(s) now lied to the Plaintiff's mother, Raeleen Woodbury, who was looking into the situation with her son **NOT** receiving adequate medical care. An attempt to cover their inadequate medical care, deliberate indifference to his medical needs. The Plaintiff's mother made these Defendant(s) aware of the Plaintiff's medical situation and was seeking to see that her son, the Plaintiff, was receiving proper medical treatment as this has gone on to long, because the Plaintiff has received numerous fractures within his left foot with no explanation. The ADA guy, Defendant Long, says they are doing all they can for him and Raeleen Woodbury said then why can't he have a food tray brought to him and why is he walking up and down flights of stairs when he should be off of his foot. Defendant Long said Man, you and I are not medically trained professionals and can't just make decisions just on him wanting them. Raeleen Woodbury corrected Defendant Long stating, you might not be medically trained, but I am (Woodbury). The warden (Defendant Falk) told Raeleen Woodbury that things doesn't happen over night. For the Plaintiff to receive a second tier housing restriction all it took was for Defendant Massenburg to give him that restriction and security would have moved into a first tier cell. Defendant Blatnick commented on the Plaintiff's injuries being old and that is why he was denied physical therapy. Raeleen Woodbury informed then, the Defendant(s) that you had no injuries to your left foot when the Plaintiff entered prison and proceeded to say that it took them till yesterday to get the Plaintiff to a specialist and then they stated they did not know who this specialist was. The Plaintiff's enters this letter from his mother Raeleen Woodbury in as **Exhibit #9** and ask this Honorable Court to review it.

22nd of April 2014; On this day the Plaintiff had another meeting with Defendant Blatnick and states that Defendant Falk and Defendant Long were also present in the phone conference call with Raeleen Woodbury the Plaintiff's mother.

SEE Exhibit #5 - Document #4

This Plaintiff told the Defendant Blatnick that the foot specialist told him that his foot should have been stabilized and he should not have been walking up the stairs, but Defendant Blatnick just made excuses to the statement that they were doing what they could. The Plaintiff raised the fact that the treatment that was given was **NOT** done in a timely fashion and that caused him to receive more injuries and subjected him to more pain and suffering that could have been avoided. Defendant Blatnick went on to state that medical could not have given him a medical lay-in and a continuous and effective pain medication without the approval from the prison administration, Defendant Falk and Defendant Long. The Plaintiff would like to point out that Defendant Falk and Defendant Long are not medically licensed and have **NO MEDICAL TRAINING AND SHOULD NOT BE MAKING MEDICAL DECISIONS**. The Plaintiff asked this Honorable Court to review this exhibit and they will see that these named Defendant(s) have showed a deliberate indifference to the Plaintiff's medical needs.

C. NATURE OF THE CASE

22nd of April 2014; The Plaintiff filed his Step 3 Grievance requesting adequate medical care, Exhibit #6 - Document #3, and this grievance was denied and it states that the Plaintiff has exhausted his administrative remedies.

The Plaintiff states that he has filed this Complaint due to the fact that he was not receiving adequate medical care and his grievance has been denied with administrative remedies exhausted, satisfying the Prison Litigation Reform Act for filing a 42 U.S.C 1983.

22nd of April 2014; The Plaintiff filed his Step 3 Grievance on Defendant Rosler for Code of Conduct and this grievance was denied and the step 3 grievance officer states that the Plaintiff did not exhaust his administrative remedies in this matter, but does not allow the Plaintiff a means to exhaust these administrative remedies. This factor that the Plaintiff grieves in this motion is **NOT** the basis for which this Plaintiff filed this complaint.

SUPPORTIVE CASE LAW

Constitution requires prison authorities to provide them with "reasonable adequate" medical. (Newman v. Alabama, 559 F.2d 283, 291 (5th Cir. 1978); accord, Hoptowit v. Ray, 682 F.2d 1237, 1246 (9th Cir. 1982); Wolfish v. Levi, 573 F.2d 118, 125 (2d Cir. 1978), rev'd on other grounds subnom. Bell v. Wolfish, 441 U.S. 520, 99 S. Ct. 1861 (1979)(1979); see also Langley v. Coughlin, 888 F.2d 252, 254 (2d Cir. 1989)(officials must provide "reasonably necessary medical care...which would be available to [the prisoner] if not incarcerated")

The fact that the care may be expensive does not excuse prison officials from providing it. (Harris v. Thigpen, 941 F.2d 1495, 1509 (11th Cir. 1991); Ancata v. Prison Health Services, Inc., 769 F.2d 700, 705 (11th Cir. 1985); Rosado v. Alameida, 349 F. Supp. 2d 1340, 1348 (S.D. Cal. 2004); Kosilek v. Maloney, 221 F. Supp. 2d 156, 161 (D. Mass. 2002); ; Yarbaugh v. Roach, 736 F. Supp. 318, 320 n.7 (D.D.C. 1990")

Myers v. Klevenhagen, 97 F.3d 91 (5th Cir. 1996); see also Hudgins v. Debruyn, 922 F. Supp. 144, 146 (S.D. Ind. 1996)(upholding requirements that many over-the-counter medications be purchased in the commissary, **except** when dispensed by medical staff "as part of a necessary treatment regimen for a serious medical condition" or "when an offender is an inpatient")

The Supreme Court has stated that "deliberate indifference to serious medical needs of prisoners constitutes the 'unnecessary and wanton infliction of pain'.....proscribed by the Eighth Amendment. (Esteele v. Gamble, 429 U.S. 97, 104, 97 S. Ct. 285 (1976); see Erickson v. Pardus, 551 U.S. 89, 94, 127 S. Ct. 2197 (2007)

Holding administrators could be held deliberately indifferent based on their knowledge of deficiencies in medical care. (Bass v. Wallenstein, 769 F.2d 1173, 1184-86 (7th Cir. 1985)).

Acts or statements of prison personnel directly demonstrate an indifferent or hostile attitude toward prisoners medical needs. (Mullen v. Smith, 783 F.2d 317, 318-19 (8th Cir. 1984) (prisoner was subjected to "ridicule and derision" in response to complaints of pain and inability to walk.)

The Plaintiff has more supportive case law, but has no room in the Complaint for entry,

D. CAUSE OF ACTION

State concisely every claim that you wish to assert in this action. For each claim, specify the right that allegedly has been violated and state all supporting facts that you consider important, including the date(s) on which the incident(s) occurred, the name(s) of the specific person(s) involved in each claim, and the specific facts that show how each person was involved in each claim. You do not need to cite specific cases to support your claim(s). If additional space is needed to describe any claim or to assert more than three claims, use extra paper to continue that claim or to assert the additional claim(s). The additional pages regarding the cause of action should be labeled "D. CAUSE OF ACTION."

1. Claim One: Violation of the Eighth Amendment of the U.S. Constitution

Supporting Facts: Protection Against Cruel and Unusual Punishment,
the infliction of pain and suffering.

Between the dates of circa September 18th, 2012 to this present date, this Plaintiff has had to endure the indignities of being abused physically, mentally, and spiritually by each and every one of these 11 (Eleven) named Defendant(s), either directly or indirectly, and this Plaintiff has had to suffer with much physical and mental trauma due to the malicious with evil intent actions by each and every one of these named Defendant(s). Due to the cruel and unusual punishment inflicted upon this Plaintiff, particularly with the pain and suffering that was inflicted upon this Plaintiff due to these named Defendant(s) showing a deliberate indifference to the Plaintiff's medical needs. This Plaintiff will have to live with the permanent and mental scarring caused by each and every one of these 11 (Eleven) named Defendant(s) in this Prisoner 42 U.S.C. 1983 action. Each and every one of these 11 (Eleven) named Defendant(s) acted with maliciously with evil intent. This Plaintiff has had to suffer with the physical and now permanent limitations placed upon him by each and every one of these 11 (Eleven) named Defendant(s). This Plaintiff has a permanent disability within in his neurological nerve system that affects his back and mainly his **left leg and his left foot**. The Plaintiff within this complaint first just had one fracture and that fracture has evolved into more fractures that could have been avoided by these named 11 (Eleven) Defendant(s) if this Plaintiff would have received adequate medical care and these 11 (Eleven) named Defendant(s) would not have shown a deliberate indifference to the Plaintiff's medical needs. This Plaintiff relies on the ability to walk, it is a major life activity and one that is essential in life and in performing daily life tasks. At this current time this Plaintiff still is not being given adequate medical care by these named Defendant(s), this Plaintiff still is not being given a continuous and effective pain medication to assist him with the pain that he endures from the nerve damage in his lower back and left leg, and this Plaintiff's left foot still is fractured and in a immobilized fixation, that of a Cam walker boot, and the Plaintiff is not receiving any pain medication for the pain. These Defendant(s) still are denying this Plaintiff the ability to receive physical therapy which was recommended by a provider, but is being denied do to it being a "pre-existing condition". The actions of these named Defendant(s) are that of cruel and unusual punishment. This Plaintiff contends that each and every one of these 11 (Eleven) named Defendant(s) has maliciously with evil intent misconstrued their official positions and these actions are detrimental and obstructive to any and all of the Plaintiff's medical needs. This Plaintiff contends that these 11 (Eleven) named Defendant(s) lack the ability to make proper judgments, or the ability of weighing present documented medical facts that would have kept this Plaintiff from the pain and suffering, cruel and unusual punishment that these 11 (Eleven) named Defendant(s) inflicted and still continue to inflict upon this Plaintiff. This Plaintiff has tried several times to receive adequate medical care, this Plaintiff has had numerous meetings with Defendant Blatnick who is the Health Service

D. CAUSE OF ACTION

.....continuation of claim.

1. Claim One: Violation of the Eighth Amendment of the U.S. Constitution
Protection Against Cruel and Unusual Punishment

Adminisrator at the Limon Correctional Facility and this Defendant Blatnick is the considered in her capacity with the medical department as the "gatekeeper", having the ability with a supervisory position to see that the Plaintiff recieved adequate medical care, but this particular Defendant Blatnick did not do so. These 11 (Eleven) named Defendant(s) cannot claim immunity. The cruel and unusual punishment being inflicted upon this Plaintiff by these 11 (Eleven) named Defendant(s) is unconstitutional. The Plaintiff contends that the 11 (Eleven) named Defendant(s) committed numerous/volumous acts and personally participated directly or indirectly, these 11 (Eleven) named Defendant(s) had the power to direct and control the alleged malicious with evil intent acts against the Plaintiff.

Amendment VIII. Excessive Bail, Fines, Punishments

Excessive bail shall not be required, nor excessive fines imposed, **nor cruel and unusual punishments inflicted.**

E. PREVIOUS LAWSUITS

Have you ever filed a lawsuit, other than this lawsuit, in any federal or state court while you were incarcerated? ___ Yes ☒ No (CHECK ONE). If your answer is "Yes," complete this section of the form. If you have filed more than one lawsuit in the past, use extra paper to provide the necessary information for each additional lawsuit. The information about additional lawsuits should be labeled "E. PREVIOUS LAWSUITS."

1. Name(s) of defendant(s) in prior lawsuit: N/A
2. Docket number and court name: N/A
3. Claims raised in prior lawsuit: N/A
4. Disposition of prior lawsuit (for example, is the prior lawsuit still pending? Was it dismissed?): N/A
5. If the prior lawsuit was dismissed, when was it dismissed and why? N/A
6. Result(s) of any appeal in the prior lawsuit: N/A

F. ADMINISTRATIVE RELIEF

1. Is there a formal grievance procedure at the institution in which you are confined?
☒ Yes ___ No (CHECK ONE).
2. Did you exhaust available administrative remedies? ☒ Yes ___ No (CHECK ONE).

G. REQUEST FOR RELIEF

State the relief you are requesting. If you need more space to complete this section, use extra paper. The additional requests for relief should be labeled "G. REQUEST FOR RELIEF."

- * This Plaintiff is requesting a trial by jury.
- * This Plaintiff is requesting a Temporary Injunction to ORDER the Defendant(s) to provide any and all proper medical care to this Plaintiff, including proper medications, in that of physical therapy. The Plaintiff seeks the Defendant(s) to provide this Plaintiff with a first tier housing restriction so this Plaintiff will not have to walk up the stairs which would restrict him from having any future physical medical injuries.
- * Declare that the acts and omissions described herein violated this Plaintiff's rights under the Constitution of the United States and the laws that govern.
- * Enter judgement in favor of the Plaintiff for nominal, compensatory, and punitive damages, those allowed by law and/or by a jury, against each Defendant(s), jointly or severally.
- * Order such additional relief as this Court may deem just and proper.
- * Plaintiff will be seeking reimbursement to all related costs here within this 1983 Claim, court cost, copies, postage, attorney fees, and any other accumulated costs.
- * The Plaintiff will be requesting a restraining order from this court, because some of these Defendant(s) are within the administrative capacity within the Limon Correctional Facility. The Plaintiff will asking for this restraining order to protect from harassment from these individuals and or the subordinate officers that these Defendant(s) within administration have control over to perform direct duties at their authority.

DECLARATION UNDER PENALTY OF PERJURY

I declare under penalty of perjury that I am the plaintiff in this action, that I have read this complaint, and that the information in this complaint is true and correct. *See* 28 U.S.C. § 1746; 18 U.S.C. § 1621.

Executed on

8-4-14
(Date)


(Prisoner's Original Signature)

CERTIFICATE OF PRISON OFFICIAL

I certify that the attached statement is an accurate copy of the inmate trust fund account statement or institutional equivalent for the past six months for the prisoner named below.

Prisoner's Name: Aaron S. Rabidue

Signature of Authorized Prison Official: _____

Date: _____

DC Form 750-01B (01/01/11)

COLORADO DEPARTMENT OF CORRECTIONS
LEGAL ACCESS PROGRAM
PHOTOCOPY REQUEST FORM

LAST NAME: Rabidue FIRST INITIAL: A DOC #: 154691
FACILITY: LCF UNIT/TIER/CELL: 6B-7-12 DATE SUBMITTED: 8-13-14

LIST THE DATE OF ANY COURT IMPOSED DEADLINE:

A signed and completed miscellaneous withdrawal slip must accompany this request.

Please list each document to be copied on separate lines and include description, number of pages you are submitting, and number of copies you are requesting. Incomplete forms will be returned or denied.

DESCRIPTION OF DOCUMENTS SUBMITTED	# PAGES SUBMITTED	# COPIES REQUESTED	# COPIES APPROVED
1 <u>6 month inmate Account statement</u>	<u>04pg</u>	<u>1</u>	<u>1</u>
2 	 	 	
3 	 	 	
4 	 	 	
5 	 	 	

OFFENDER SIGNATURE (REQUIRED) [Signature]

DO NOT WRITE BELOW THIS LINE - OFFICIAL USE ONLY

OFFENDER AVAILABLE FUNDS: \$ <u>✓</u>	CHARGE TO ACCOUNT: \$ <u>1.00</u>
DATE RECEIVED AT SENDING FACILITY: <u>AUG 13 2014</u>	NUMBER OF PAGES RECEIVED: <u>4 from DocNet</u>
DATE RECEIVED IN PROCESSING LAW LIBRARY: <u>AUG 13 2014</u>	
NUMBER OF PAGES RECEIVED IN PROCESSING LAW LIBRARY: <u>4</u>	
DATE COMPLETED: <u>AUG 13 2014</u>	
DOC EMPLOYER/CLERK PHOTOCOPIER INITIALS: <u>SLR</u>	
METER END: <u>58466</u>	
MINUS	METER BEGIN: <u>58461</u>
TOTAL COPIES: <u>4</u>	
MINUS	ADMINISTRATIVE COPIES: <u>1</u>
TOTAL COPIES TO OFFENDER: <u>5</u>	DATE DENIED: <u> </u>

Your request has been **DENIED** _____ in whole _____ in part for the following reasons:

- ☐ Your request form was not properly completed (Missing required signature, full cell location, facility, unit, tier, cell, DOC #, date, etc.)
- ☐ The material you have submitted does not meet program definitions of legal material, as described in AR 750-01.
- ☐ Your photocopy request exceeds the page limit established by the legal access program (see attached). Also see posted photocopy policies.
- ☐ Regardless of your ability to pay, you will be supplied only the required number of copies as dictated by court rule or statute. Your request is in excess of those requirements (see attached). Also see posted photocopy policies.
- ☐ You have not submitted the documents to be copied.
- ☐ You must submit a completed miscellaneous withdrawal ticket with your request, regardless of indigence status.
- ☐ Attachments/exhibits to a document must be submitted with the original document, even if the original is not being copied.
- ☐ Your account is in arrears for at least \$500. See AR 750-01 for additional information.
- ☐ The legal access program will not copy ARs, IAs, OMs, or material contained in the law library, even as attachments/exhibits.
- ☐ The legal access program will not copy transcripts, incomplete documents, unsigned documents, altered documents, and/or blank forms.
- ☐ The legal access program will not copy non-original documents, previously-copied documents, incoming correspondence, or documents (account statements, mittimus, etc.), grievances, COPD appeals; *except* as exhibits attached to an original pleading being filed with the court or attached to a letter to a judge or attorney of record. Your pleading **must** include a statement referring to the attached exhibits in order for them to be copied.
- ☐ You may bring your request into compliance and resubmit.

Other _____

Attachment "B"



154691 - RABIDUE, AARON S

LIMON CF, LCF/UNIT6



Printed 08/13/2014 at 2:32 pm by szhein

Criteria — Date Start: , End:

STATEMENT OF ACCOUNT ACTIVITY

Statement Period: 02/13/2014 - 08/13/2014

Date:Time	Description	Tran Amt	Balance	Available	Encumbrances			Available	FC
				Before W/H	Type	Amount	Total	Balance	
02/13/2014 00:00	Beginning Balance	0.00	152.10	152.10			5.38	146.72	
02/14/2014 10:00	Canteen #10252894 Hold	0.00		152.10	CANTEEN	6.00	11.38	140.72	
02/19/2014 06:56	Canteen Ord#10252894	-6.00	146.10	146.10		0.00	5.38	140.72	LF
02/21/2014 09:43	Canteen #10270730 Hold	0.00		146.10	CANTEEN	78.36	83.74	62.36	
02/24/2014 08:56	Canteen Ord#10270730	-78.36	67.74	67.74		0.00	5.38	62.36	LF
02/27/2014 23:22	JPAY CREDIT	191.30	259.04	259.04	MANDATORY	38.26	43.64	215.40	LF
02/28/2014 00:38	RESTITUTION-10CR355	-43.64	215.40	215.40		0.00	0.00	215.40	LF
02/28/2014 11:18	Canteen #10283779 Hold	0.00		215.40	CANTEEN	58.23	58.23	157.17	
03/02/2014 23:23	JPAY CREDIT	20.00	235.40	235.40	MANDATORY	4.00	62.23	173.17	LF
03/03/2014 06:53	Canteen Ord#10283779	-58.23	177.17	177.17		0.00	4.00	173.17	LF
03/03/2014 16:25	1 CUSTODIAN	6.90	184.07	184.07	MANDATORY	1.38	5.38	178.69	LF
03/04/2014 23:25	MEDICAL EMERGENCY	-5.00	179.07	179.07		0.00	5.38	173.69	LF
03/07/2014 11:54	Canteen #10298743 Hold	0.00		179.07	CANTEEN	60.02	65.40	113.67	
03/11/2014 06:50	Canteen Ord#10298743	-60.02	119.05	119.05		0.00	5.38	113.67	LF
03/14/2014 07:46	Canteen #10312107 Hold	0.00		119.05	CANTEEN	66.25	71.63	47.42	
03/18/2014 07:00	Canteen Ord#10312107	-66.25	52.80	52.80		0.00	5.38	47.42	LF
03/21/2014 07:58	Canteen #10330783 Hold	0.00		52.80	CANTEEN	42.08	47.46	5.34	
03/24/2014 16:15	XEROX-DEBIT	-4.00	48.80	48.80		0.00	47.46	1.34	LF

03/25/2014 09:30	LIGHT BULBS DEBIT	-0.30	48.50	48.50		0.00	47.46	1.04	LF
03/26/2014 06:59	Canteen Ord#10330783	-42.08	6.42	6.42		0.00	5.38	1.04	LF
03/31/2014 01:31	RESTITUTION- 10CR355	-5.38	1.04	1.04		0.00	0.00	1.04	LF
04/01/2014 10:37	POSTAGE- DEBIT	-1.82	-0.78	0.00		0.00	0.00	0.00	LF
04/01/2014 10:39	POSTAGE- DEBIT	-0.98	-1.76	0.00		0.00	0.00	0.00	LF
04/01/2014 23:24	JPAY CREDIT	40.00	38.24	38.24	MANDATORY	8.00	8.00	30.24	LF
04/04/2014 16:08	1 CUSTODIAN	6.90	45.14	45.14	MANDATORY	1.38	9.38	35.76	LF
04/11/2014 07:53	Canteen #10368607 Hold	0.00		45.14	CANTEEN	22.00	31.38	13.76	
04/11/2014 11:40	Canteen #10374911 Hold	0.00		45.14	CANTEEN	12.90	44.28	0.86	
04/14/2014 07:30	Canteen Ord#10368607	-22.00	23.14	23.14		0.00	22.28	0.86	LF
04/14/2014 16:04	XEROX-DEBIT	-0.75	22.39	22.39		0.00	22.28	0.11	LF
04/14/2014 23:26	MEDICAL EMERGENCY	-5.00	17.39	17.39		0.00	22.28	-4.89	LF
04/17/2014 06:39	Canteen Ord#10374911	-12.90	4.49	4.49		0.00	9.38	-4.89	LF
04/17/2014 09:17	HYGIENE ITEMS DEBIT	-0.35	4.14	4.14		0.00	9.38	-5.24	LF
04/27/2014 23:25	WESTERN UNION CREDIT	50.00	54.14	54.14	MANDATORY	10.00	19.38	34.76	LF
04/30/2014 00:39	RESTITUTION- 10CR355	-19.38	34.76	34.76		0.00	0.00	34.76	LF
05/01/2014 17:37	1 CUSTODIAN	6.90	41.66	41.66	MANDATORY	1.38	1.38	40.28	LF
05/01/2014 23:29	MEDICAL APPOINT	-3.00	38.66	38.66		0.00	1.38	37.28	LF
05/02/2014 07:41	Canteen #10410937 Hold	0.00		38.66	CANTEEN	21.25	22.63	16.03	
05/05/2014 11:39	Canteen Ord#10410937	-21.25	17.41	17.41		0.00	1.38	16.03	LF
05/09/2014 08:03	Canteen #10426316 Hold	0.00		17.41	CANTEEN	5.51	6.89	10.52	
05/09/2014 23:23	JPAY CREDIT	50.00	67.41	67.41	MANDATORY	10.00	16.89	50.52	LF
05/13/2014 06:29	Canteen Ord#10426316	-5.51	61.90	61.90		0.00	11.38	50.52	LF
05/16/2014 07:29	Canteen #10444923 Hold	0.00		61.90	CANTEEN	20.00	31.38	30.52	
05/19/2014 10:29	XEROX-DEBIT	-2.00	59.90	59.90		0.00	31.38	28.52	LF
05/20/2014	Canteen	-20.00	39.90	39.90		0.00	11.38	28.52	LF

06:24	Ord#10444923								
05/29/2014 23:23	JPAY CREDIT	50.00	89.90	89.90	MANDATORY	10.00	21.38	68.52	LF
05/30/2014 00:39	RESTITUTION- 10CR355	-21.38	68.52	68.52		0.00	0.00	68.52	LF
05/30/2014 07:34	Canteen #10468763 Hold	0.00		68.52	CANTEEN	27.92	27.92	40.60	
06/02/2014 15:29	PHOTOS - DEBIT	-6.00	62.52	62.52		0.00	27.92	34.60	LF
06/02/2014 15:36	1 CUSTODIAN	6.90	69.42	69.42	MANDATORY	1.38	29.30	40.12	LF
06/03/2014 06:33	Canteen Ord#10468763	-27.92	41.50	41.50		0.00	1.38	40.12	LF
06/06/2014 08:08	Canteen #10482872 Hold	0.00		41.50	CANTEEN	20.00	21.38	20.12	
06/10/2014 06:34	Canteen Ord#10482872	-20.00	21.50	21.50		0.00	1.38	20.12	LF
06/13/2014 13:44	POSTAGE- DEBIT	-3.40	18.10	18.10		0.00	1.38	16.72	LF
06/20/2014 08:32	Canteen #10516865 Hold	0.00		18.10	CANTEEN	16.00	17.38	0.72	
06/23/2014 11:03	Canteen Ord#10516865	-16.00	2.10	2.10		0.00	1.38	0.72	LF
06/26/2014 07:53	Canteen #10526013 Hold	0.00		2.10	CANTEEN	0.72	2.10	0.00	
06/27/2014 16:44	MEDICAL- CREDIT 5-1-14	3.00	5.10	5.10		0.00	2.10	3.00	LF
06/30/2014 00:40	RESTITUTION- 10CR355	-1.38	3.72	3.72		0.00	0.72	3.00	LF
06/30/2014 12:04	Canteen Ord#10526013	-0.72	3.00	3.00		0.00	0.00	3.00	LF
07/03/2014 08:03	Canteen #10536641 Hold	0.00		3.00	CANTEEN	3.00	3.00	0.00	
07/03/2014 17:10	1 CUSTODIAN	7.59	10.59	10.59	MANDATORY	1.52	4.52	6.07	LF
07/08/2014 06:47	Canteen Ord#10536641	-3.00	7.59	7.59		0.00	1.52	6.07	LF
07/11/2014 09:29	Canteen #10558232 Hold	0.00		7.59	CANTEEN	6.00	7.52	0.07	
07/14/2014 07:33	Canteen Ord#10558232	-6.00	1.59	1.59		0.00	1.52	0.07	LF
07/25/2014 23:25	JPAY CREDIT	50.00	51.59	51.59	MANDATORY	10.00	11.52	40.07	LF
07/31/2014 01:41	RESTITUTION- 10CR355	-11.52	40.07	40.07		0.00	0.00	40.07	LF
08/01/2014 09:07	Canteen #10596261 Hold	0.00		40.07	CANTEEN	39.87	39.87	0.20	
08/02/2014 23:27	JPAY CREDIT	20.00	60.07	60.07	MANDATORY	4.00	43.87	16.20	LF
08/05/2014 06:33	Canteen Ord#10596261	-39.87	20.20	20.20		0.00	4.00	16.20	LF

08/06/2014 08:45	1 CUSTODIAN	7.59	27.79	27.79	MANDATORY	1.52	5.52	22.27	LF
08/06/2014 13:46	HOBBY-DEBIT	-1.00	26.79	26.79		0.00	5.52	21.27	LF
08/08/2014 08:25	Canteen #10609515 Hold	0.00		26.79	CANTEEN	17.49	23.01	3.78	
08/08/2014 10:47	Canteen #10612261 Hold	0.00		26.79	CANTEEN	3.70	26.71	0.08	
08/11/2014 10:30	POSTAGE- DEBIT	-1.89	24.90	24.90		0.00	26.71	-1.81	LF
08/11/2014 11:34	Canteen Ord#10609515	-17.49	7.41	7.41		0.00	9.22	-1.81	LF
08/13/2014 10:26	XEROX-DEBIT	-1.00	6.41	6.41		0.00	9.22	-2.81	LF
08/13/2014 14:32	Ending Balance	0.00	6.41	6.41		0.00	9.22	-2.81	

Total Deposits: 517.08
Total Withdrawals: 662.77

Account Information as of 08/13/2014 14:32			
Status: ACTIVE	Current Balance: \$6.41	Total Money In Hold: \$0.00	
	Total Reserved/Encumbered: \$9.22	Available Balance: \$-2.81	

Statement Information

Date:Time reflects the transaction date and time of deposits and withdrawals to the account during the statement period.

Description is a brief memorandum describing the transaction.

Tran Amt is the amount of the transaction; deposits are shown as a positive number, withdrawals are shown as a negative number.

Balance is the actual balance of the account. All deposits and withdrawals are added and subtracted from the 'Balance'. If an account has a negative balance an 'Available Balance' will be derived according to AR 200-2.

Available Before W/H is the derived available balance where 50% of deposits are made available when the actual account balance prior to deposit is a negative amount (Ref. AR 200-2). This amount is derived prior to mandatory withholding.

Encumbrances are amounts placed on hold pending further processing by the banking system, such as when a canteen order is posted or a deposit is subject to mandatory garnishment rules. The amount, type of hold, and a total (as of the Date:Time) is reported.

Available Balance shows monies available for withdrawal at the end of the transaction. The time precision for all transactions is to the minute; encumbrance total may duplicate for multiple transactions if the transaction date and time are identical.

FC (Facility Code) is the offender's facility assignment at the date and time the transaction was posted to the account, not the originating source of the transaction.

NOTICE: This statement may not include most recent Withdrawals or Deposits.

Printed 08/13/2014 at 2:32 pm by szhein



EXHIBIT LIST

- * Exhibit #1 - Medical Records from the Colorado Department of Corrections
- * Exhibit #2 - X-ray reports - Colorado Department of Corrections
- * Exhibit #3 - Medical Records from 2006 spinal cord injury
- * Exhibit #4 - Grievances on not receiving adequate medical care for chronic pain
- * Exhibit #5 - Affidavits
- * Exhibit #6 - Grievances for adequate medical care, fractured left foot
- * Exhibit #7 - Grievances on Defendant Rosler
- * Exhibit #8 - Medical Records from when the Plaintiff went to Denver Health in Denver Colorado
- * Exhibit #9 - Letter from Plaintiff's mother - Raeleen Woodbury
- * Exhibit #10 - Copy of letter sent to Defendant Blatnick
- * Exhibit #11 - Medication Change copied from prison news station

These exhibits supra have documents attached to them in numbering order.

- * Exhibit #12 - Administrative Regulation #100-01
- * Exhibit #13 - Administrative Regulation #100-19
- * Exhibit #14 - Administrative Regulation #700-01
- * Exhibit #15 - Administrative Regulation #700-02
- * Exhibit #16 - Administrative Regulation #700-10
- * Exhibit #17 - Administrative Regulation #700-15
- * Exhibit #18 - Administrative Regulation #700-18
- * Exhibit #19 - Administrative Regulation #1450-01
- * Exhibit #20 - Administrative Regulation #700-21
- * Exhibit #21 - Declaration for Prose Capacity

Patient Chart

Rabidue, Aaron

Date Printed: 07/13/11
614-26-2536 Sex: M Age: 22 DOB: 05/06/1989

Progress Notes

08/13/09 : 10:10am

Progress Note

MYC

TITLE:

Several diagnoses, including abscess behind left ear and also history of back injury with paresthesias in bilateral lower extremities.

SUBJECTIVE: Patient here today with several concerns. First of all, he is here to follow-up on the abscess which was drained several days ago behind his left ear. Patient notes that the wick fell out, he's been using gentle moist heat to the area. He's been able to express a scant amount of purulent drainage from the area. He's been taking his antibiotics and tolerating them without difficulty. He notes that the edema has significantly improved and the pain has basically resolved. He has no fevers, chills or systemic symptoms and feels that this is significantly on the mend.

He has one other concern to discuss today. He notes that he has a past history of an assault when he was 17 years old. He was stabbed with a pair of scissors on the right flank area, he was hospitalized for 8 days. He believes that the injury was about an inch to inch and a half deep and was very narrow. He was told that it was about an inch off from his spine. It was clearly on the right side. He has developed some lower extremity paresthesias. He notes that he developed them immediately upon being stabbed. He was kicked several times, he believes in the back although he is not sure. He notes that for the last 3 years he has been having numbness, tingling and some neuropathic discomfort in his lower extremities, although describes the left as being significantly worse than the right. He notes difficulty balancing on the toes on the left and states that he's had trouble with strength in his left leg ever since the initial injury 3 years ago. He believes that x-rays were taken. Records have been requested but are not yet available for review. He has not had any recent follow-up. He has never had follow-up or evaluation by a neurosurgeon. He is not sure if he's had an MRI of the back. He never had physical therapy. He was treated with multiple pain medications which made him sick. He was also treated with Baclofen and Neurontin, which may have been somewhat helpful although he never took them without the narcotics so he is not sure whether these medications contributed to making him feel sick as well.

He denies other ongoing health issues and is not currently taking any regular medications other than the antibiotics prescribed two days ago for his abscess.

He has no known drug allergies.

PHYSICAL EXAM: He is well appearing, in no distress. The abscess is basically resolved. Edema and induration have resolved. The skin behind his ear is still a little bit loose but there is no clear pocket in the area. The wick is out and the area has started to scab over. He has no cervical lymph adenopathy. He does have an old healed, approximately 1 inch wound on the right side of his back, about 1 to 2 inches to the right of midline. He does have weakness with plantar flexion at the left ankle and is unable to walk on his toes on the left side or to get up on his toes on the left side. Exam is otherwise generally nonfocal.

ASSESSMENT:

Follow-up abscess, resolving. Also, history of a stab wound to the right side of his back with neuropathic symptoms in the left lower extremity and possible blunt trauma to his back at the same time.

PLAN: He will continue with his antibiotics, he'll continue with the moist heat to the area behind his left ear and will express gently any residual pus. He'll follow-up if he has any signs of worsening induration or recurrent infection. At this time he seems to be healing well.

Printed using Practice Partner®

Phib. #1 - Doc. #1

EXHIBIT 1

MEDICAL RECORDS FROM THE COLORADO DEPARTMENT OF CORRECTIONS

Patient Chart

Date Printed: 07/13/11

Rabidue, Aaron

614-26-2536 Sex: M Age: 22 DOB: 05/06/1989

The history is somewhat confusing with regards to his previous back injury. The stab injury was clearly on the right but he clearly has significant neuropathic symptoms on the left. He certainly could have sustained a kick or a blunt trauma to the left side of his back, which would affect the left lower extremity. At this time he is offered a prescription for Baclofen and/or Neurontin, which have been helpful in the past. He declines them because he had some side-effects while on them, although it is not clear whether these medications were responsible for the side-effects or whether it was the narcotics causing the side-effects. He is offered an MRI, physical therapy referral or a neurosurgical referral. He declines all of these at this time due to cost-effectiveness issues. He is given information for the CACP program, will try to get on this program, and will follow-up if and when he gets on it to pursue additional interventions as indicated and desired. Follow-up sooner with any significant change. At this time he is 3 years out from his injury and things are at least stable, though as young as he is and as significant as his symptoms are, it certainly warrants additional workup and intervention sooner rather than later.

Maria Y. Chansky, MD

MYC/cr

D: 08/13/2009 13:56:50

T: 08/17/2009 16:47:46

SIGNED BY MARIA Y CHANSKY, MD (MYC) 08/18/2009 10:14AM

Exhibit #1 ~ Doc. #2

DEPARTMENT OF CORRECTIONS AMBULATORY HEALTH RECORD

Printed By: SICOTTE, TRUDY A

Printed at: 09/18/2012 17:00:25

Encounter#: 3001521 @ LF

APPOINTMENT

Page 1 of 1

154691	RABIDUE, AARON S	Facility:	LF	LU:	LCF/UNIT3	A	1	11	T
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SUBJECTIVE

I/M visit for kite for previous back injury and pain. I/M states he was paralyzed from a stabbing in the middle of his back in 2006. I/M states he learned to walk again from a paraplegic state. I/M states he was in DRDC when he came in and did not want any medications because of his religion. I/M states now he has chronic pain that radiates down L leg with a jumping action.

Temperature:	98.0	Pulse:	75	Weight:	193
Respiratory:	18	BP:	129 / 69		

Vitals Taken: PULSE OXSYM=94.00;

PLANS / ORDERS

Allergies: No Known Allergies

Please send for medical records.

I/M verbalizes understanding of kite system.

NEW - Rx#: 2987145 Drug Name: IBUPROFEN - 600MG Alternate Name: MOTRIN Rx Date: 09/18/2012 Disc Date: 11/17/2012 Rx Instructions: TAKE 1 TABLET(S) BY MOUTH TWICE DAILY AS NEEDED.

Medical Level: Changed from: 1 Qualifier: PERMANENT to 1 Qualifier: PERMANENT

OBJECTIVE

I/M has never sent for his records so form was given and I/M made the form out. First, he wanted me to get the information from his grandmother and I said no you will make out the form Easy as hospital/doctor/date.

I/M request flexeril, lyrica and narcotics.

Ibuprofen was ordered.

During exam I/M is taking his shirt off w/exuberance and readily and spontaneously moving all extremities in a wild manner. No problem w/rom in any form.

General alert no apparent distress.

VSS pulse 75 bp 129/69.

Cardiac rrr o murmur.

Lungs are cta bilaterally.

He does have a faded scar in the middle of back.

Extremities no edema full rom. No limp noticed.

Neurologic CN11-CNX11 intact reflexes are 2+ bilaterally U and L extremities.

ASSESSMENT

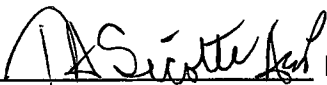
338.21 - CHRONIC PAIN DUE TO TRAUMA

99212 - OFFICE/OUTPATIENT VISIT, EST

Datetime:
09/18/2012 16:48

Providers: SICOTTE, TRUDY

PA/NP/RD

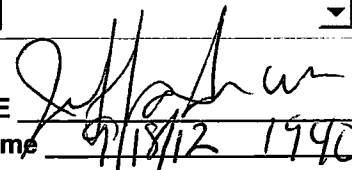


PHYSICIAN

Datetime: 09/18/2012 16:48

Provider: SICOTTE, TRUDY

NURSE



Date Time

9/18/12 1940

420109
DEPARTMENT OF CORRECTIONS
AMBULATORY HEALTH RECORD

Encounter#: 3044848 @ LF

Page 1 of 1

154691	RABIDUE, AARON S	Facility:	LF	LU:	LCF/UNIT3	A	1	11	B
SUBJECTIVE Active Medication(s): IBUPROFEN - 600MG - TAKE 1 TABLET(S) BY MOUTH TWICE DAILY AS NEEDED. I/M here to request neurontin. I have nerve damage in R leg. I/M also has psorias and would like tca cream.		Temperature: 98.2 Pulse: 90 Weight: 196 Respiratory: 18 BP: 144 / 79 Vitals Taken: PULSE OXSYM=96.00;							
OBJECTIVE 23 y/o Caucasion male w/BMI 22. I/M was stabbed 6 yrs ago and went to the hospital where he was under observation for a 1..5 cm tranverse approximately 2 cm right at the midline at the T12 level. ON exam I/M has some atrophy of R calf. I/M has full strength in R leg and 4/5 in Left as he did when he left the hospital 6 yrs ago. I/M cannot completely pivot the L ankle but walks without a limp. I/M upon questioning is performing 2 hrs of strenuous exercise a day.		PLANS / ORDERS Allergies: No Known Allergies I/M verbalizes understanding of plan of care, will place for PT and neurontin. I/M verbalizes understanding of kite system. I/M verbalizes understanding of absorbase for treatment of psorias but will prescribe TCA for sparing use. Outside Consultation#: 164228 - Level: 1 MONTH - Request: PT - Location: LIMON CF Medical Level: Changed from: 1 Qualifier: PERMANENT to 1 Qualifier: PERMANENT NEW: Kite#: 2219887 For MEDICAL - RETURN TO CLINIC - APPOINTMENT - RTC F/U AFTER MEDICAL RECORDS REVIEWED, PT AND ANSWER FROM PHARMACY FOR NEURONTIN.							
ASSESSMENT 338.21 - CHRONIC PAIN DUE TO TRAUMA 99212 - OFFICE/OUTPATIENT VISIT EST									
		Datetime: 11/08/2012 13:40		Providers: SICOTTE, TRUDY					

NURSE 14 [Signature]
Date/Time 11/08/12 11:00

Attachment B

COLORADO DEPARTMENT OF CORRECTIONS
NON-FORMULARY DRUG REQUEST

Run Date: 11/08/2012 14:27:25

Rx#:

Offender's Name: RABIDUE, AARON S

DOC#: 154691

Facility: LIMON CF

MEDICAL CONDITION being treated: nerve damage

CRITICAL MEDICATION? Yes ☐ No ☒

If yes, Provide rationale why this medication is considered critical:

Previous remedies, canteen, formulary medications used (include length of time, dosage and compliance):

None, I/M is requesting neurontin and will not take any thing else after talking with him.

Rationale why a non-formulary drug is necessary (include/attach clinical documentation and evidence based medicine):

I/M performs 2 hr of strenuous exercise a day here at Limon. But he was stabbed 6 yr ago never f/u for pt or f/u w/any Md. On day of discharge day eight of his hospital stay, I/M displayed 4/5 strength in R leg and as he had on admit 5/5 in L leg. I/M does have some atrophy of L calf. No surgical intervention was needed and I/M was kept for observation. The stab wound was 1.5 cm transverse stab wound, approximately 2 cm right at the midline at the T12 level.

Supporting medical information (including physical exam, labs, diagnostic testing, imaging, etc). Use additional page as needed.

I/M VSS, and I/M strolls down the hall without a limp. I/M tells this np that he has no feeling in L L L. I asked if he has no feeling why do you need neurontins? He states he was on baclofen, neurontin, Elavil and Novco.

Is this a consultant recommendation:

Yes ☐ No ☒

Have alternate RX's been discussed:

Yes ☒ No ☐

Please include consultant response/report and contact phone number:

Date: 11/8/2012 Provider: SICOTTE, TRUDY A
Drug/Strength: GABAPENTIN - 100MG - 16382
Alternate Name: NEURONTIN
SIG: Length (in days): 180
take 1 tablet bid x 180 days.

Nurse Signature

Date

PA/NP Signature

Date

Printed Name:

Reviewing Officer Comments:

☒ Approved as is☐ Modified to☐ Denied

Signature of Medical Officer

Date

Scanned
11/08/12
Jai

nl - Inhouse RN 11/08/12
Trudy Ann Lucette 11/8/12
Printed Name: Trudy A. Secotte

Exhibit #1 - Dec. #5

Colorado Department of Corrections

Page 1 of 1

Run Date: 11/08/2012 14:30

Appt#: 164228

Consultation Report Form

REQUESTED

DOC #: 154691 Name: RABIDUE, AARON S Facility: LCF/UNIT3 Date Initiated: 11/08/2012
Gender: MALE Security: CLOSE SSN: 614-26-2536 DOB: 05/06/1989
Level: 1 MONTH Tele-Med: NO Provider: SICOTTE, TRUDY A, NP SDD: 05/11/2028
Request: PT Auth #: Number of Visits: 1 PED: 01/21/2024
Location: LIMON CF Appt Dt: Specialist: DH #:

Diagnosis and requested evaluation or care:

1. CLINICAL INFORMATION:

Subjective: Active Medication(s): IBUPROFEN - 600MG - TAKE 1 TABLET(S) BY MOUTH TWICE DAILY AS NEEDED.

I/M states he could not afford PT after his hospitalization for observation, no surgery, after being stabbed 1.5 cm tranverse approximately 2 cm. R at the midline at the T 12 level.

ICD

338.21	CHRONIC PAIN DUE TO TRAUMA

CPT

99212	OFFICE/OUTPATIENT VISIT EST

Provider Signature:

Electronically Signed

Date: 11/08/2012 14:30

Provider Name:

SICOTTE, TRUDY A, NP

tasicott

Exhibit #1 - Doc. #6

Please do not discuss dates of the follow-up appointments with the offender.

Insurance: Correctional Health Partners Phone: (866) 362-1374 FAX: (866) 362-1375

Please send all dictated reports to: Clinical Services - Dept of Corrections - 2862 S. Circle Colorado Springs, CO 80906

Or FAX to your Scheduler @ 719-226-4500

PHYSICIAN HEALTH PARTNERS
1515 Arapahoe St., Suite #300, Tower 1
Denver, CO 80202

UTILIZATION REVIEW
Phone: (720) 612-6700
Fax: (303) 605-1545

Request ID: 009-10864262

Event ID# 0010864503

Date: 11/9/2012

IPA: CDC

Priority: ROUTINE

Patient Name: RABIDUE, AARON

Health Plan: DEPARTMENT OF CORRECTIONS

Patient Sex: M

Plan ID: 154691

DOB: 5/6/1989

Group: 1P

Address: CDOC

LIMON CORRECTIONAL FACIL

CANON CITY, CO 81212

PCP: LIMON CORRECTIONAL FACILITY

Phone:

PCP Phone: 719-775-9221

Requesting: LIMON CORRECTIONAL
FACILITY

Referred To: SELECT PHYSICAL THERAPY - CASTLE ROCK

Phone: 719-775-9221

Phone:

Fax: 719-775-7651

Fax:

Contact:

Facility:

Request Type: THERAPY - PHYSICAL

Diagnosis: 338.21 CHRONIC PAIN DUE TO TRAUMA

Past Treatment Comments:

CDOC REQUEST 164228: Hosp Name: LIMON CF (LF) Clin Name: PT (40) Specialist: SELECT PT Provider: TRUDY SICOTTE (B857)
CDOC REQUEST 1. CLINICAL INFORMATION: Subjective: Active Medication(s): IBUPROFEN - 600MG - TAKE 1 TABLET(S) BY MOUTH
TWICE DAILY AS NEEDED. I/M states he could not afford PT after his hospitalization for observation, no surgery, after being stabbed 1.5
cm transverse approximately 2 cm. R at the midline at the T 12 level.

Auth Status: PENDED

Decision Date: 11/15/2012

Procedure: Description:

Appr. From: Appr. To: Units:

97001 PHYSICAL THER EVAL

11/08/2012 11/09/2012 0 VISIT(S)

Approval Specifications / Comments:

I/M with atrophy of R calf. I/M has full strength in R leg 5/5 and 4/5 in L leg. I/M was stabbed on R flank area at 14 but presumably sustained other injury to L during altercation. This altercation took place 7 yr ago.

TAE Sicotte Rm

This referral is not a guarantee of payment.
Coverage will be determined based on eligibility and availability of remaining benefits at the time of the service.

Phibit #1- Doc #7

PH: Fax

11/20/2012 3:46:08 PM PAGE 2/003

Fax Server

PHYSICIAN HEALTH PARTNERS
1515 Arapahoe St., Suite #300, Tower 1
Denver, CO 80202

UTILIZATION REVIEW
Phone: (720) 612-6700
Fax: (303) 605-1545

Request ID: 009-10864262

Event ID# 0010864503

Date: 11/9/2012

IPA: CDC

Priority: ROUTINE

Patient Name: RABIDUE, AARON

Health Plan: DEPARTMENT OF CORRECTIONS

Patient Sex: M

Plan ID: 154691

DOB: 5/6/1989

Group: 1P

Address: CDOC

LIMON CORRECTIONAL FACIL

CANON CITY, CO 81212

PCP: LIMON CORRECTIONAL FACILITY

Phone:

PCP Phone: 719-775-9221

Requesting: LIMON CORRECTIONAL
FACILITY

Referred To: SELECT PHYSICAL THERAPY - CASTLE ROCK

Phone: 719-775-9221

Phone:

Fax: 719-775-7651

Fax:

Contact:

Facility:

Request Type: THERAPY - PHYSICAL

Diagnosis: 338.21 CHRONIC PAIN DUE TO TRAUMA

Past Treatment Comments:

CDOC REQUEST 164228: Hosp Name: LIMON CF (LF) Clin Name: PT (40) Specialist: SELECT PT Provider: TRUDY SICOTTE (B857)
CDOC REQUEST 1. CLINICAL INFORMATION: Subjective: Active Medication(s): IBUPROFEN - 600MG - TAKE 1 TABLET(S) BY MOUTH
TWICE DAILY AS NEEDED. I/M states he could not afford PT after his hospitalization for observation, no surgery, after being stabbed 1.5
cm tranverse approximately 2 cm. R at the midline at the T 12 level.

Auth Status: DENIED MEDICAL NECESSITY

Decision Date: 11/12/2012

Procedure: Description:

Appr. From: Appr. To: Units:

97001 PHYSICAL THER EVAL

11/08/2012 05/07/2013 0 VISIT(S)

Approval Specifications / Comments:

11.16.12: Remains denied, information provided does not support medical need for PT JAM

Phib #1- Doc #8

EMERGENCY

Page 1 of 1

DEPARTMENT OF CORRECTIONS AMBULATORY HEALTH RECORD

154691	RABIDUE, AARON S	Facility:	LF	LU:	LCF/UNIT2	B	2	14
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SUBJECTIVE Active Medication(s): CETIRIZINE - 10 MG - TAKE 1 TABLET(S) BY MOUTH ONCE DAILY IN THE EVENING FOR ALLERGY (SYMPTOMS).; PSYLLIUM FIBER - 0.52 GM - TAKE 2 CAPSULE(S) BY MOUTH TWICE DAILY.; TRIAMCINOLONE OINT - 0.1% - APPLY TO AFFECTED AREA ONCE DAILY IN THE EVENING (UNTIL SYMPTOMS) RELIEVED. USE SPARINGLY INTERMITTENTLY "My buddy told me I might have gout. This started on Sunday and has gotten worse." OBJECTIVE Offender declared an emergency for L foot pain. He was observed to limp and favoring the left side. Able to get on exam table without assistance. Denies recent injury. Denies any recent slips, trips or falls. Describes old injury to back that left deficiencies to L leg (sensory and strength). Upon visual exam, minimal swelling is noted to top aspect of foot. No discoloration noted. Cap refill <2 seconds. Foot is warm and dry to touch. Full ROM with some pain (6/10)---states that pain and swelling are worse at night. Describes pain that is on the outer aspect of foot-->underside to arch. VS stable. Consulted with PA Massenburg. VO: Motrin 600mg PO BID x 7 days; Ice w/permit BID x 4 days; ace wrap w/HCA x7days. RTC for re-eval with nursing on 2/4/14. chart to provider for review. Offender shown how to apply the ace wrap. Noted to limp less noticeably and stated that his ankle feels more supported. Encouraged to RTC if symptoms worsen. Given pt education sheet and instructed to elevate foot when not walking/exercise/etc. Offender verbalized understanding and agrees with TX plan. ASSESSMENT see nursing protocol: sprains, strains, muscle aches dated 1/31/14	Temperature: 98.4 Pulse: 74 Weight: 203 Respiratory: 16 BP: 137 / 76 Vitals Taken: PULSE OXSYM=94.00;
	PLANS / ORDERS Allergies: No Known Allergies RTC if symptoms worsen or as needed Consulted with PA Massenburg. VO: Motrin 600mg PO BID x 7 days; Ice w/permit BID x 4 days; ace wrap w/HCA x7days. RTC for re-eval with nursing on 2/4/14. chart to provider for review. Offender verbalized understanding and agrees with TX plan.
	NEW - VERBAL - Rx#: 3208865 Drug Name: IBUPROFEN - 600MG Alternate Name: MOTRIN Rx Date: 01/31/2014 Disc Date: 02/07/2014 Rx Instructions: TAKE 1 TABLET(S) BY MOUTH TWICE DAILY FOR PAIN.
	NEW: Kite#: 2692750 For NURSING - RETURN TO CLINIC - APPOINTMENT - RECHECK OF L FOOT PAIN

Datetime: 01/31/2014 12:06	Providers: BLATNICK, NICOLE
--------------------------------------	------------------------------------

PA/NP/RD

PHYSICIAN

NURSE

Datetime: 01/31/2014 12:06

Provider: BLATNICK, NICOLE

Date Time

1/31/14 13:25

AR Form 700-34A (09/15/12)

COLORADO DEPARTMENT OF CORRECTIONS

Health Care Appliance (HCA) Contract

Avon Rabidue 154691 UF
Name DOC # Facility

I am being issued: 3" ace wrap
Equipment type Product / Model Size

Serial/ID # Model # on this date 1/31/14

☐ Permanent Issue ☒ Temporary Issue Return date for temporary Issue 2/7/14

I understand that I share in the responsibility to keep the health care appliance listed above in good working order by using it only for its intended purpose.

I will not allow other offenders to use this health care appliance. If I allow others to use this equipment, my use of the health care appliance may be limited, restricted, or removed.

I understand that I must report any issues concerning the repair of this health care appliance to Clinical Services through the request for sick call system. CDOC is responsible to repair or replace the equipment within 60 days of receipt of my sick call request indicating there is a problem with the equipment.

I understand that, if the preponderance of the evidence shows that I altered, cut on, damaged, or destroyed this equipment, I will be responsible for the replacement cost of the equipment. In this event, I may request replacement of the same equipment at my own cost. Arrangements for payment will need to be made in advance. If I am unable to pay the cost of replacement, that cost will be charged to my inmate account.

I understand that if I misuse, alter, cut on, or otherwise damage or destroy this health care appliance, I will face COPD charges for "Misuse of Clinical Services," II(33), and "Damage to Property," II(2).

I understand that if I use this health care appliance inappropriately and my inappropriate use of such equipment results in a COPD violation, my use of the assigned health care appliance may be limited, restricted, or removed.

Avon Rabidue
Offender Signature

N. Blotnick 1/31/14
DOC Employee/Contract Worker Signature and Date

Original: Medical File
cc: Facility Property Officer
Offender
HQ CLS ADA Scheduler

Fax# 719-226-4565

Attachment "A"
Page 1 of 1

Exhibit 1 - Document #9A

1-31-14

Nicole give me this.

**COLORADO DEPARTMENT OF CORRECTIONS
CLINICAL SERVICES
PATIENT INFORMATION SHEET FOR:
SPRAINS AND STRAINS / MUSCLE ACHES**

A sprain is an injury to a ligament (which holds a bone to a bone). A strain is an injury to a muscle and to the tendon which attaches the muscle to a bone. Both can be very painful and take a few weeks to heal, depending on how big an injury they were at first. Most muscle aches are caused by overuse of that muscle, straining it, or not warming up by stretching before muscle use. Follow the instructions of your health provider and below, and you will soon be on the mend.

1. Take it easy! Since you had microscopic tearing of the fibers, you had microscopic bleeding in the area. It will take time for the body to replace the injured cells. If you rest and elevate the affected limb, it will reduce the swelling, make it less painful, and hasten healing. Using the limb a lot will only aggravate the injury. As the pain subsides, your activity may increase.

2. If you were given an anti-inflammatory medication, it will take time to build the level up in your blood. The medication will reduce the swelling. Don't miss a dose and you will have quicker results. They will allow you to relax and heal.

3. You may have been prescribed a brace or crutches. The brace will help your joint feel more stable. The crutches will take the weight off the affected limb, so it doesn't swell more. The crutches should fit under your armpits with two inches to spare; place body weight on your hands as you swing through.

4. If your health provider has prescribed exercises, do them faithfully and as directed. These will strengthen your joint and pull the swelling out as well. When recovered from muscle aches/strains, be sure to gently stretch muscles in the future before strenuous use.

5. Notify Clinical Services if:

- ☞ You are not healing in the time frame you have been given.
- ☞ The medication causes you to have indigestion, nausea or vomiting.
- ☞ Your pain level or swelling is increasing instead of steadily decreasing.
- ☞ You have any numbness in/or below the site of the injury.
- ☞ You are running a fever.

Exhibit #1 - Document #9B

DEPARTMENT OF CORRECTIONS AMBULATORY HEALTH RECORD

Printed By: ROSLER, TINA M

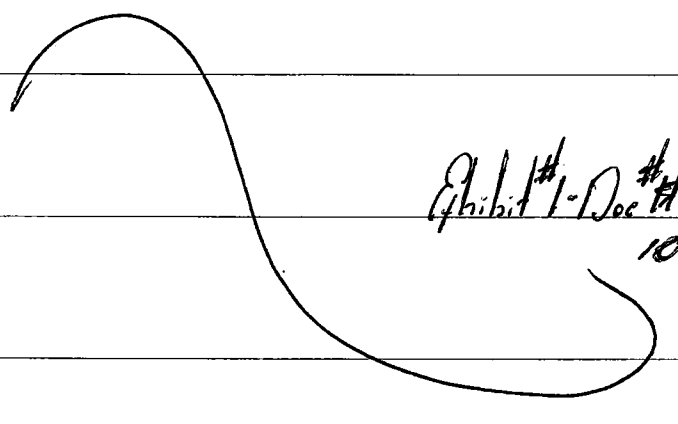
Printed at: 02/04/2014 22:18:00

Encounter#: 3478923 @ LF

FOLLOW-UP

Page 1 of 1

154691	RABIDUE, AARON S	Facility:	LF	LU:	LCF/UNIT2	B	2	14
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SUBJECTIVE Active Medication(s): CETIRIZINE - 10 MG - TAKE 1 TABLET(S) BY MOUTH ONCE DAILY IN THE EVENING FOR ALLERGY (SYMPTOMS).; IBUPROFEN - 600MG - TAKE 1 TABLET(S) BY MOUTH TWICE DAILY FOR PAIN.; PSYLLIUM FIBER - 0.52 GM - TAKE 2 CAPSULE(S) BY MOUTH TWICE DAILY.; TRIAMCINOLONE OINT - 0.1% - APPLY TO AFFECTED AREA ONCE DAILY IN THE EVENING (UNTIL SYMPTOMS) RELIEVED. USE SPARINGLY INTERMITTENTLY	Temperature: Pulse: Weight: Respiratory: BP: /
	PLANS / ORDERS Allergies: No Known Allergies
	Verbal order: A. Massenburg, PA / T. Rosler RN Tylenol 1000mg PO BID X 4 days. 8 packets given.
	Instructed to continue to keep the foot wrapped and elevate as much as possible.
OBJECTIVE Offender to clinic for follow up of left foot pain. Walks with a limp, favoring the left. Foot is wrapped in ace wrap. States it continues to hurt 8/10 and that the pain keeps him up at night. Pedal pulse present and strong. Cap refill less than 3 seconds. Provider Massenburg consulted. VO for X-ray next on site. Tylenol packets (8) given with instruction to take 1 packet at noon and one before bed which will alternate with the Motrin already ordered. Instructed to continue to keep the foot wrapped and elevate as much as possible. RTC on Friday for f/u. Verbalized understanding and agreed to plan.	RTC on Friday for f/u.
	Verbalized understanding and agreed to plan.
	NEW: Kite#: 2696507 For NURSING - X-RAYS - APPOINTMENT - LEFT FOOT AND ANKLE
	NEW: Kite#: 2696509 For NURSING - RETURN TO CLINIC - APPOINTMENT - F/U LEFT FOOT PAIN
ASSESSMENT	
	Datetime: 02/04/2014 22:11
	Providers: ROSLER, TINA
	(Empty box for additional provider information)

PA/NP/RD

Datetime: 02/04/2014 22:11

PHYSICIAN

Provider: ROSLER, TINA

NURSE

Date/Time

 1-1-14
 10

DEPARTMENT OF CORRECTIONS
AMBULATORY HEALTH RECORD

Printed By: ROSLER, TINA M

Printed at: 02/11/2014 15:26:42

Encounter#: 3485812 @ LF

FOLLOW-UP

Page 1 of 1

154691	RABIDUE, AARON S	Facility:	LF	LU:	LCF/UNIT2	B	2	14
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SUBJECTIVE
Active Medication(s): CETIRIZINE - 10 MG - TAKE 1 TABLET(S) BY MOUTH ONCE DAILY IN THE EVENING FOR ALLERGY (SYMPTOMS).;
PSYLLIUM FIBER - 0.52 GM - TAKE 2 CAPSULE(S) BY MOUTH TWICE DAILY.;
TRIAMCINOLONE OINT - 0.1% - APPLY TO AFFECTED AREA ONCE DAILY IN THE EVENING (UNTIL SYMPTOMS) RELIEVED. USE SPARINGLY INTERMITTENTLY

OBJECTIVE
Offender to clinic for f/u left foot pain. X-ray results are not yet here. Contact was made with the agency that does our x-rays and indicated that results would not be available until at least Thursday of this week. Area remains slightly swollen. Pedal pulse strong. Cap refill less than 3 seconds. States it is numb from spinal cord injury but that he feels "intense pain inside the foot". 8/10. Pain medication (Motrin and Tylenol) expired. Consulted with Massenburg PA-C who gave a verbal order for 2 Tylenol 3 BID X 2 days. First dose given in clinic. Instructed to return tomorrow am for another dose. Continue to elevate as much as possible and avoid excess exercise. Offender will have RTC after X-Ray results are received. Already has an appointment scheduled with provider for Wednesday for chronic nerve complaints. Instructed to contact medical if condition worsens. Verbalized understanding and agreed to plan.

ASSESSMENT

PLANS / ORDERS
☒ Allergies: No Known Allergies
☒ Verbal order A. Massenburg PA-C / T. Rosler RN Two Tylenol 3 PO BID X 2 days. Crush and Float. First dose given.
☒ Keep foot elevated as much as possible.
☒ Contact medical if s/s worsen.
☒ Keep follow-up provider appointments.
☒ Chart to provider for review.
☒ CLOSED: Kite#: 2696507 For NURSING - X-RAYS - APPOINTMENT - LEFT FOOT AND ANKLE

Datetime: 02/11/2014 14:48
Providers: ROSLER, TINA

Phibisi #1-Dec. #14

PA/NP/RD

PHYSICIAN

Datetime: 02/11/2014 14:48

Provider: ROSLER, TINA

NURSE

Date Time

T. Rosler
2/11/14 @ 1525

DEPARTMENT OF CORRECTIONS
AMBULATORY HEALTH RECORDPrinted By: MASSENBURG, ALVIN E
Printed at: 02/12/2014 16:20:58
Encounter#: 3487576 @ LF

APPOINTMENT

Page 1 of 1

154691	RABIDUE, AARON S	Facility:	LF	LU:	LCF/UNIT2	B	2	14
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SUBJECTIVE I/M presents with c/o of pain Left Lower leg due to nerve damage. At age of 17, inmate was stabbed in the back resulting in some Left lower extremity weakness. He was able to begin walking again but has had decreased sensation in Left foot on the outside and left lower leg on the outside. He also has problems moving his foot up/down and left to right. It turns in when he walks. Reports it is worst now with stinging pain and numbness in left foot and leg. Active Medication(s): CETIRIZINE - 10 MG - TAKE 1 TABLET(S) BY MOUTH ONCE DAILY IN THE EVENING FOR ALLERGY (SYMPTOMS).; PSYLLIUM FIBER - 0.52 GM - TAKE 2 CAPSULE(S) BY MOUTH TWICE DAILY.; TRIAMCINOLONE OINT - 0.1% - APPLY TO AFFECTED AREA ONCE DAILY IN THE EVENING (UNTIL SYMPTOMS) RELIEVED. USE SPARINGLY INTERMITTENTLY	Temperature:	97.6	Pulse:	61	Weight:	204
	Respiratory:	18	BP:	147	/	73
	Vitals Taken: PULSE OXSYM=93.00;					

OBJECTIVE General: Gait is unassisted with some guarding when stepping down. Left foot turns medially when walking. NAD, alert and oriented Left Lower leg: calf muscle atrophy, no pain to palpation of calf muscle, decreased touch sensation lateral lower leg L5, Muscle strength with extension 5/5 Left Foot: Mild inflammation, ROM reduced with flexion and extension, Muscle strength against resistance flexion/extension 2/5, Unable to fully flex/extend 5th, 4th and 3rd digits. Reduced Inversion/Eversion. Decreased touch sensation along S1 Left foot. Foot inverts with walking. ASSESSMENT 338.21 - CHRONIC PAIN DUE TO TRAUMA - Advised that PT consult would be submitted and Neurontin trial for pain in Left Foot. Still awaiting x-ray results of Left Foot. Will provide those to I/M when received. 99212 - OFFICE/OUTPATIENT VISIT EST	PLANS / ORDERS Allergies: No Known Allergies NEW - Rx#: 3214223 Drug Name: GABAPENTIN - 400MG Alternate Name: NEURONTIN Rx Date: 02/12/2014 Disc Date: 06/12/2014 Rx Instructions: TAKE 2 CAPSULE(S) BY MOUTH TWICE DAILY FOR PAIN. Outside Consultation#: 182984 - Level: UPTO 2 MNTHS - Request: PT - Location: LIMON CF					

Datetime:	02/12/2014 16:02	Providers:	MASSENBURG, ALVIN E
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PAIN/PRD

PHYSICIAN

NURSE

Datetime: 02/12/2014 16:02

Provider: MASSENBURG, ALVIN E

Date Time

Colorado Department of Correction

Consultation Report Form

Run Date: 02/12/2014 16:20

Appt#: 182984

REQUESTED

DOC #: 154691 Name: RABIDUE, AARON S Facility: LCF/UNIT2 Date Initiated: 02/12/2014
 Gender: MALE Security: CLOSE SSN: 614-26-2536 DOB: 05/06/1989
 Level: UPTO 2 MNTHS Tele-Med: NO Provider: MASSENBURG, ALVIN E, PA SDD: 12/11/2027
 Request: PT Auth #: Number of Visits: 1 PED: 01/21/2024
 Location: LIMON CF Appt Dt: Specialist: DH #:

Diagnosis and requested evaluation or care:**1. CLINICAL INFORMATION:**

Subjective: I/M presents with c/o of pain Left Lower leg due to nerve damage. At age of 17, inmate was stabbed in the back resulting in some Left lower extremity weakness. He was able to begin walking again but has had decreased sensation in Left foot on the outside and left lower leg on the outside. He also has problems moving his foot up/down and left to right. It turns in when he walks. Reports it is worst now with stinging pain and numbness in left foot and leg.

Active Medication(s): CETIRIZINE - 10 MG - TAKE 1 TABLET(S) BY MOUTH ONCE DAILY IN THE EVENING FOR ALLERGY (SYMPTOMS).;
 PSYLLIUM FIBER - 0.52 GM - TAKE 2 CAPSULE(S) BY MOUTH TWICE DAILY.;
 TRIAMCINOLONE OINT - 0.1% - APPLY TO AFFECTED AREA ONCE DAILY IN THE EVENING (UNTIL SYMPTOMS) RELIEVED. USE SPARINGLY INTERMITTENTLY

Objective: General: Gait is unassisted with some guarding when stepping down. Left foot turns medially when walking. NAD, alert and oriented
 Left Lower leg: calf muscle atrophy, no pain to palpation of calf muscle, decreased touch sensation lateral lower leg L5, Muscle strength with extension 5/5
 Left Foot: Mild inflammation, ROM reduced with flexion and extension, Muscle strength against resistance flexion/extension 2/5, Unable to fully flex/extend 5th, 4th and 3rd digits. Reduced Inversion/Eversion. Decreased touch sensation along S1 Left foot. Foot Inverts with walking.

Request Physical Therapy**Assessment:**

338.21 - CHRONIC PAIN DUE TO TRAUMA - Advised that PT consult would be submitted and Neurontin trial for pain in Left Foot. Still awaiting x-ray results of Left Foot. Will provide those to I/M when received.
 99212 - OFFICE/OUTPATIENT VISIT EST

ICD		CPT	
338.21	CHRONIC PAIN DUE TO TRAUMA	99212	OFFICE/OUTPATIENT VISIT EST

Provider Signature:	Electronically Signed	Date: 02/12/2014 16:20
Provider Name:	MASSENBURG, ALVIN E, PA	aemassen

Exhibit #1 - Doc. #13

Please do not discuss dates of the follow-up appointments with the offender.

Insurance: Correctional Health Partners Phone: (866) 362-1374 FAX: (866) 362-1375

Please send all dictated reports to: Clinical Services - Dept of Corrections - 2862 S. Circle Colorado Springs, CO 80906

Or FAX to your Scheduler @ 719-226-4500

DEPARTMENT OF CORRECTIONS
AMBULATORY HEALTH RECORD

ADMINISTRATIVE

Page 1 of 1

Printed By: MASSENBURG, ALVIN E

Printed at: 02/14/2014 11:45:31

Encounter#: 3489806 @ LF

154691	RABIDUE, AARON S	Facility:	LF	LU:	LCF/UNIT2	B	2	14
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SUBJECTIVE

I/M presents for review of Left foot/ankle x ray.
Hx of previous Left foot fracture of unknown etiology. I/M states it could have been when his foot was run over by car.

Active Medication(s): CETIRIZINE - 10 MG - TAKE 1 TABLET(S) BY MOUTH ONCE DAILY IN THE EVENING FOR ALLERGY (SYMPTOMS).;
GABAPENTIN - 400MG - TAKE 2 CAPSULE(S) BY MOUTH TWICE DAILY FOR PAIN.;
PSYLLIUM FIBER - 0.52 GM - TAKE 2 CAPSULE(S) BY MOUTH TWICE DAILY.;
TRIAMCINOLONE OINT - 0.1% - APPLY TO AFFECTED AREA ONCE DAILY IN THE EVENING (UNTIL SYMPTOMS) RELIEVED. USE SPARINGLY INTERMITTENTLY

OBJECTIVE

X-ray findings: No evidence of acute ankle fracture, some elevation of the cortex and mild expansion of the mid diaphysis of the 5th metatarsal, If patient has hx of 5th toe fracture, this may represent healing response.

Advised rest and cool compress prn. Will add Salsalate as a trial medication for pain control. I/M to kite prn. Discussed the unpredictable nature of healing process and future pain/inflammation of left foot.

ASSESSMENT

V68.89 - ENCOUNTERS OTH SPEC ADMIN PRPOS OTH
88325 - COMPREHENSIVE REVIEW OF DATA

Temperature: Pulse: Weight:
Respiratory: BP: /

PLANS / ORDERS

Allergies: No Known Allergies

NEW - Rx#: 3215262 Drug Name: SALSALATE - 750MG Alternate Name: DISALCID Rx Date: 02/14/2014 Disc Date: 04/15/2014 Rx Instructions: TAKE 1 TABLET(S) BY MOUTH TWICE DAILY AS NEEDED FOR PAIN.

Datetime: 02/14/2014 11:38

Providers: MASSENBURG, ALVIN E

PA/NP/RD

PHYSICIAN

Datetime: 02/14/2014 11:38

Provider: MASSENBURG, ALVIN E

NURSE

DateTime

[Signature]
2/14/14 3:00

Page 1 of 1

PA/NP/RD [Signature] PHYSICIAN _____ NURSE [Signature]
 Datetime: 03/04/2014 14:45 Provider: MASSENBURG, ALVIN E Date Time 3/4/14 1930

DEPARTMENT OF CORRECTIONS
AMBULATORY HEALTH RECORD

ADMINISTRATIVE

Page 1 of 1

154691	RABIDUE, AARON S	Facility:	LF	LU:	LCF/UNIT2	B	2	214	1S
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SUBJECTIVE

I/M with c/o of Left foot pain. States he stood up about 1 week ago and heard a pop. Experienced immediate pain. He described the pain as sharp. States that yesterday the foot did not hurt but today it is hurting.

Active Medication(s):

- 01) CETIRIZINE - 10 MG - TAKE 1 TABLET(S) BY MOUTH ONCE DAILY IN THE EVENING FOR ALLERGY (SYMPTOMS).
02) GABAPENTIN - 400MG - TAKE 2 CAPSULE(S) BY MOUTH TWICE DAILY FOR PAIN.
03) NAPROXEN - 500MG - TAKE 1 TABLET(S) BY MOUTH TWICE DAILY FOR PAIN.
04) PSYLLIUM FIBER - 0.52 GM - TAKE 2 CAPSULE(S) BY MOUTH TWICE DAILY.
05) TRIAMCINOLONE OINT - 0.1% - APPLY TO AFFECTED AREA ONCE DAILY IN THE EVENING (UNTIL SYMPTOMS) RELIEVED.
USE SPARINGLY INTERMITTENTLY

OBJECTIVE

General: Ambulates with no assistance, slight limp Left leg, states that yesterday the foot did not hurt but today it is hurting.
Objective: Left Foot X - ray results. Acute fracture of the 2nd metatarsal. Sub acute-chronic fracture of proximal 4th metatarsal and a chronic fracture of the 5th metatarsal. Alignment anatomic
Passive ROM of Metatarsals WNL, active plantar flexion limited due to pain complaint, very mild inflammation, pedal pulse WNL.
Foot is warm not hot to touch.

Advised to wear Post-Op Shoe x 3 weeks with followup x-ray.
Ortho consult requested. X-ray reports included

ASSESSMENT

V68.89 - ENCOUNTERS OTH SPEC ADMIN PRPOS OTH
825.20 - 825.20 - CLOSED FRACTURE UNSPEC BONE FOOT -
2ND METATARSAL
88325 - COMPREHENSIVE REVIEW OF DATA

Temperature: Pulse: Weight:
Respiratory: BP: /

PLANS / ORDERS

Allergies: No Known Allergies

D/C - Rx#: 3223101 Drug Name: NAPROXEN - 500MG Alternate Name:
NAPROSYN Rx Date: 03/04/2014 Disc Date: 03/11/2014 Rx Instructions:
TAKE 1 TABLET(S) BY MOUTH TWICE DAILY FOR PAIN.

NEW - Rx#: 3226527 Drug Name: ACETAMINOPHEN - 500MG Alternate
Name: TYLENOL Rx Date: 03/11/2014 Disc Date: 03/18/2014 Rx
Instructions: TAKE 2 TABLET(S) BY MOUTH TWICE DAILY AS NEEDED
FOR PAIN.

Outside Consultation#: 184117 - Level: 1 MONTH - Request:
ORTHOPEDIC - Location: DENVER HEALTH

Datetime:	Providers:	MASSENBURG, ALVIN E
03/11/2014 14:09		

PA/NP/RD

PHYSICIAN

NURSE

Datetime: 03/11/2014 14:09

Provider: MASSENBURG, ALVIN E

DateTime

Yaslee R
3/11/14 @ 2:00

Colorado Department of Corrections

Run Date: 03/11/2014 15:19

Appt#: 184117

Consultation Report Form

REQUESTED

DOC #: 154691 Name: RABIDUE, AARON S Facility: LCF/UNIT2 Date Initiated: 03/11/2014
 Gender: MALE Security: CLOSE SSN: 614-26-2536 DOB: 05/06/1989
 Level: 1 MONTH Tele-Med: NO Provider: MASSENBURG, ALVIN E, PA SDD: 12/01/2027
 Request: ORTHOPEDIC Auth #: Number of Visits: 1 PED: 01/21/2024
 Location: DENVER HEALTH Appt Dt: Specialist: DH #:

Diagnosis and requested evaluation or care:

1. CLINICAL INFORMATION: I/M with c/o of Left foot pain. States he stood up about 1 week ago and heard a pop. Experienced immediate pain. He described the pain as sharp.

Subjective: Active Medication(s):

01) CETIRIZINE - 10 MG - TAKE 1 TABLET(S) BY MOUTH ONCE DAILY IN THE EVENING FOR ALLERGY (SYMPTOMS).
 02) GABAPENTIN - 400MG - TAKE 2 CAPSULE(S) BY MOUTH TWICE DAILY FOR PAIN.
 03) NAPROXEN - 500MG - TAKE 1 TABLET(S) BY MOUTH TWICE DAILY FOR PAIN.
 04) PSYLLIUM FIBER - 0.52 GM - TAKE 2 CAPSULE(S) BY MOUTH TWICE DAILY.
 05) TRIAMCINOLONE OINT - 0.1% - APPLY TO AFFECTED AREA ONCE DAILY IN THE EVENING (UNTIL SYMPTOMS) RELIEVED. USE SPARINGLY INTERMITTENTLY

Objective: Left Foot X - ray results. Acute fracture of the 2nd metatarsal. Sub acute-chronic fracture of proximal 4th metatarsal and a chonic fracture of the 5th metatarsal. Alignment anatomic
 Passive ROM of Metatarsal WNL, active plantar flexion limited due to pain complaint, very mild inflammation, pedal pulse WNL.
 Foot is warm nohot to touch.

Advised to wear Post-Op Shoe x 3 weeks with followup x-ray.
 Ortho consult requested. X-ray reports included

825.20 - 825.20 - CLOSED FRACTURE UNSPEC BONE FOOT - 2ND METATARSAL

ICD		CPT	
V68.89	ENCOUNTERS OTH SPEC ADMIN PRPOS OTH	88325	COMPREHENSIVE REVIEW OF DATA
825.20	CLOSED FRACTURE UNSPEC BONE FOOT		

Provider Signature:	Electronically Signed	Date: 03/11/2014 15:19
Provider Name:	MASSENBURG, ALVIN E, PA	aemassen

Exhibit #1 - Doc. #17

Please do not discuss dates of the follow-up appointments with the offender.

Insurance: Correctional Health Partners Phone: (866) 362-1374 FAX: (866) 362-1375

Please send all dictated reports to: Clinical Services - Dept of Corrections - 2862 S. Circle Colorado Springs, CO 80906

Or FAX to your Scheduler @ 719-226-4500

Colorado Department of Corrections

Run Date: 03/11/2014 15:19

Appt#: 184117

Consultation Report Form

REQUESTED

DOC #: 154691 Name: RABIDUE, AARON S Facility: LCF/UNIT2 Date Initiated: 03/11/2014
 Gender: MALE Security: CLOSE SSN: 614-26-2536 DOB: 05/06/1989
 Level: 1 MONTH Tele-Med: NO Provider: MASSENBURG, ALVIN E, PA SDD: 12/01/2027
 Request: ORTHOPEDIC Auth #: Number of Visits: 1 PED: 01/21/2024
 Location: DENVER HEALTH Appt Dt: Specialist: DH #:

Diagnosis and requested evaluation or care:

1. CLINICAL INFORMATION: I/M with c/o of Left foot pain. States he stood up about 1 week ago and heard a pop. Experienced immediate pain. He described the pain as sharp.

Subjective: Active Medication(s):

- 01) CETIRIZINE - 10 MG - TAKE 1 TABLET(S) BY MOUTH ONCE DAILY IN THE EVENING FOR ALLERGY (SYMPTOMS).
 02) GABAPENTIN - 400MG - TAKE 2 CAPSULE(S) BY MOUTH TWICE DAILY FOR PAIN.
 03) NAPROXEN - 500MG - TAKE 1 TABLET(S) BY MOUTH TWICE DAILY FOR PAIN.
 04) PSYLLIUM FIBER - 0.52 GM - TAKE 2 CAPSULE(S) BY MOUTH TWICE DAILY.
 05) TRIAMCINOLONE OINT - 0.1% - APPLY TO AFFECTED AREA ONCE DAILY IN THE EVENING (UNTIL SYMPTOMS) RELIEVED. USE SPARINGLY INTERMITTENTLY

Objective: Left Foot X - ray results. Acute fracture of the 2nd metatarsal. Sub acute-chronic fracture of proximal 4th metatarsal and a chronic fracture of the 5th metatarsal. Alignment anatomic
 Passive ROM of Metatarsal WNL, active plantar flexion limited due to pain complaint, very mild inflammation, pedal pulse WNL.
 Foot is warm nohot to touch.

Advised to wear Post-Op Shoe x 3 weeks with followup x-ray.
 Ortho consult requested. X-ray reports included

825.20 - 825.20 - CLOSED FRACTURE UNSPEC BONE FOOT - 2ND METATARSAL

ICD		CPT	
V68.89	ENCOUNTERS OTH SPEC ADMIN PRPOS OTH	88325	COMPREHENSIVE REVIEW OF DATA
825.20	CLOSED FRACTURE UNSPEC BONE FOOT		

Provider Signature:	Electronically Signed	Date: 03/11/2014 15:19
Provider Name:	MASSENBURG, ALVIN E, PA	aemassen

Exhibit #1 - Doc. #18

Please do not discuss dates of the follow-up appointments with the offender.

Insurance: Correctional Health Partners Phone: (866) 362-1374 FAX: (866) 362-1375

Please send all dictated reports to: Clinical Services - Dept of Corrections - 2862 S. Circle Colorado Springs, CO 80906

Or FAX to your Scheduler @ 719-226-4500

DEPARTMENT OF CORRECTIONS

AMMILATORY HEALTH RECORD

ADMINISTRATIVE

Page 1 of 1

154691	RABIDUE, AARON S	Facility:	LF	LU:	LCF/UNIT2	B	2	214	1S
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SUBJECTIVE Active Medication(s): 01) GABAPENTIN - 400MG - TAKE 2 CAPSULE(S) BY MOUTH TWICE DAILY FOR PAIN. 02) PSYLLIUM FIBER - 0.52 GM - TAKE 2 CAPSULE(S) BY MOUTH TWICE DAILY.	Temperature: Pulse: Weight: Respiratory: BP: /
	PLANS / ORDERS Allergies: No Known Allergies
	Offender verbalized understanding but is not confident of the outcome.
	Pain management will be discussed with PA Massenburg. Also will ensure that a follow up appointment is set with xrays if needed per provider.
OBJECTIVE Offender was brought back the clinic to be informed of findings in regards to outside consults and pain management options. Offender was told about PT consult that was denied based on "pre-existing condition." He verbalized frustration regarding this decision and wanted to know if it would always be considered "pre-existing" regardless of length of time in DOC. I told him I didn't have that answer of what it might be in the future. He was informed of Ortho consult that is still pending. He wanted to know what that entails; it was explained that they are the bone specialists and can determine the next course of treatment. He describes a lot of frustration with on going pain—nothing has worked for him from Salsalate to Tylenol #3. He believes that it is deliberate on the part of medical. He also is worried about how vulnerable he is with showing a weakness in regards to his foot. He describes increased "shooting electrical pain from his foot all the way to his gluteal muscles." He expresses interest in finding a solution to dealing with this issue as well. It was discussed that he can pursue an outside opinion based on AR 700-21. A later appointment would be set for specifics if he wished; he declined. He was upfront about the intent to pursue legal actions. Pain management will be discussed with PA Massenburg. Also will ensure that a follow up appointment is set with xrays if needed per provider. Offender verbalized understanding but is not confident of the outcome.	chart to provider for review
	ASSESSMENT V68.89 - ENCOUNTERS OTH SPEC ADMIN PRPOS OTH 88325 - COMPREHENSIVE REVIEW OF DATA
Datetime: 03/24/2014 15:25 Providers: BLATNICK, NICOLE ☐ ☐ ☐	

PA/NP/RD

PHYSICIAN

NURSE

Datetime: 03/24/2014 15:25

Provider: BLATNICK, NICOLE

DateTime

3/24/14P.1650

Printed at: 03/25/2014 13:47:08

Encounter#: 3531445 @ LF

Page 1 of 1

DateTime

Datetime: 03/25/2014 12:55

62 of 69

Page 1 of 1

DateTime

Fixed 3/15/14

Exhibit #1-
Dec. 4/22

DateTime

KUCHER, VALENTINA	▼
	▼
	▼

DEPARTMENT OF CORRECTIONS

AMBULATORY HEALTH RECORD

Printed By: DANG, HONG N

Printed at: 04/14/2014 22:33:24

Encounter#: 3553308 @ LF

EMERGENCY

Page 1 of 1

154691	RABIDUE, AARON S	Facility:	LF	LU:	LCF/UNIT2	B	2	214	1S
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SUBJECTIVE Active Medication(s): 01) PSYLLIUM FIBER - 0.52 GM - TAKE 2 CAPSULE(S) BY MOUTH TWICE DAILY. Offender declared medical emergency with c/o foot pain. Offender reported that this morning as he was bearing full weight on his left foot; his foot bent in and he got pain at left foot arch. it was sharp and throbbing pain; level was 7/10 constantly. When I concerned if offender had any liver issues, he stated with me and officers that "No. No Hep C or B, but I wish I had. It is cool." I told him, "Believe me. It is not as you thought. You won't want to get it." Offender concerned about x-ray "I want to take x-ray this Wednesday because I may get a new injury." I replied that, "You will have an appointment with a foot specialist soon this week. He/she will check on you. If he/she thinks that you need to take x-ray; then we will do it. I don't want to schedule for you to take x-ray now because we don't want to schedule then cancel it due to your appointment with a foot specialist." When I gave offender 6 packages of Tylenol and instructed him to take one package a time; three times a day only. Offender said, "Here is enough for me only this evening." I said seriously that "You can't take all at the same time. ONE package a time only and 6 hours apart." Offender said nothing and left. OBJECTIVE Offender was escorted into clinic. Offender ambulated by his own power and with partial weight on left foot. No acute distress noted. Offender was A&O. ACE wrap and surgical boot were used. Offender's VSs: T99.1; P92; R20; BP 149/81; SpO2 96%. Upon assessment, no edema or erythema noted at this time at left foot arch compared with the right one. Capillary refill <2 seconds. Offender could move left toes and foot. Both Posterior tibialis and dorsalis pedis pulses were present and strong. Skin at affected areas: normal warm. When I was doing in the evening medline, I saw offender Rabidue walked by out side the medline. He walked fine; no acute distress noted. ASSESSMENT See Nursing protocol: Sprains, strains, muscle aches on 04/14/14.	Temperature: 99.1 Pulse: 92 Weight: Respiratory: 20 BP: 149 / 81 Vitals Taken: PULSE OXSYM=96; PLANS / ORDERS Allergies: No Known Allergies Given 6 packages of Tylenol and instructed to take one package a time, 6 hours apart as needed for pain. Instructed to avoid overuse his left foot. Verbalized understanding and left without any issues. NEW: Kite#: 2773587 For NURSING - RETURN TO CLINIC - APPOINTMENT - F/U PAIN AT LEFT FOOT ARCH Exhibit #1 - Dec. #24
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Datetime: 04/14/2014 21:40	Providers: DANG, HONG
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PA/NP/RD

Datetime: 04/14/2014 21:40

PHYSICIAN

Provider: DANG, HONG

NURSE

Date Time

4/14/14 22:35

COLORADO DEPARTMENT OF CORRECTIONS: CLINICAL SERVICES

Sec 2

NURSING PROTOCOL: SPRAINS, STRAINS, MUSCLE ACHES

NAME: Ralidue DATE: 4/14/14 TIME: 1535
 DOC #: 154691 FACILITY: LCF AGE: 34 CURRENT MEDS: fibex
 ALLERGIES: NKA SEX: ☒ M ☐ F
 PERTINENT MEDICAL HISTORY: _____ PAST SURGICAL HISTORY: _____

Subjective: Onset (date): 04/14/14 (time): 0800 Pain Scale (0-10): 0 1 2 3 4 5 6 7 8 9 10
 Quality of pain: ☐ Burning ☐ Stabbing ☒ Throbbing (none) (moderate) (severe)
☒ Sharp ☐ Dull ☐ Pressure
☒ Constant ☐ Intermittent Radiation: ☒ No ☐ Yes, to _____
 Location: Right foot arch
 Color of Skin over affected area: ☒ Normal ☐ Reddish ☐ Bruising
 Signs and Symptoms: ☐ Swelling ☐ Numbness ☐ Tingling ☐ Hemorrhage
☒ Difficulty bearing weight ☐ Inability to bear weight
☐ Difficulty using extremity ☐ Inability to use extremity
Objective: Vital Signs: Temp. 99.1 Pulse 92 Resp. 20 B/P 149/81 SpO2 96%
 Capillary refill of affected limb: ☐ N/A or 12 seconds
 Pulse of affected limb: ☐ N/A ☐ brachial- ☐ radial- ☐ ulnar ☐ femoral
☐ popliteal- ☒ posterior tibialis- 2+ ☒ dorsalis pedis- 2+
 Reflexes of lower extremity: ☐ N/A ☐ Patellar: ☐ Abnormal ☒ Normal
☐ Achilles: ☐ Abnormal ☒ Normal
 Reflexes of upper extremity: ☐ N/A ☐ Brachioradial ☐ Abnormal ☐ Normal
 Check all reflexes: ☐
 S/S Inflammation: ☒ Warm to touch ☐ Cool to touch ☐ Edema ☐ Redness ☐ Bruising
 Amt. of Swelling: ☒ None ☐ Small ☐ Large, describe: _____
 Amt. of Bleeding: ☒ None ☐ Small ☐ Large, describe: _____

Describe ROM (range of motion): _____

Assessment Decision:

Findings Requiring IMMEDIATE Referral:

- ☐ Unable to bear wt/move extremity
- ☐ Possible fracture
- ☐ Lg. amt. of swelling or bleeding into area
- ☐ Sensory deficit or abnormal reflexes
- ☐ Circulatory deficit below injury

Findings NOT Requiring IMMEDIATE Referral:

- ☒ Finger or toe injury with no obvious deformity to provider at next work day

Plan:

(Pick the appropriate actions from the list below)

- ☐ Refer to: _____ Date: - Time: _____
- ☒ Transported to emergency room at: _____ (location) by _____ (van, amb.) Time: _____
- ☒ Area stabilized in functional position with a splint. already using splint
- ☒ Sling or Crutches given and instructed about appropriate use.
- ☐ Ice area X 10 minutes with protective barrier to prevent frostbite.
- ☒ Un-Aspirin (Acetaminophen) 500 mg, 2 tabs PO TID for 24 hours.
- ☐ Ibuprofen 400 mg QID for 24 hours.
- ☐ Verbal Order for: _____
- ☐ Patient education/information sheet given

Nurse's Signature: H. Dane RN Date: 04/14/14 NP/PA/MD/DO Signature: V. Kuchler, MD Date: 4.15.14

If based upon your collection of the above data you feel a provider's judgment is required or you have questions about how to proceed, you must consult with a mid-level provider or physician while the patient is still on-site.

August 2011

Reference: Lippincott's Nursing Procedures 5th Edition

EXHIBIT #2

**X-RAY REPORTS ON THE PLAINTIFF
X-RAYS WERE TAKEN IN THE CARE OF THE
COLORADO DEPARTMENT OF CORRECTIONS**



Pueblo Radiological Group
P.O. Box 9347
Pueblo, CO, 81008
T 719 584-7415
F 719 542-7019

Patient Name:	Rabidue, Aaron	Accession:	163D73DOC
MRN:	154691DOC	DoB:	May 6, 1989
Physician:	Massenburg, Alvin	Sex:	Male
Exam:	73610-ANKLE (COMPLETE)	Exam Date:	February 5, 2014 09:10
		Report Status:	Final

Findings

Reporting MD: Moore, Mark
Dictation Time: Not available
Transcriptionist: Mathews, Kristina
Transcription Date: February 10, 2014 08:38

LEFT ANKLE AND LEFT FOOT ACCESSIONS 163D73DOC AND 163D74DOC ARE COMBINED IN THIS REPORT

CLINICAL HISTORY: Pain.

FINDINGS: X-rays of the left foot demonstrated expansion of the cortex of the 5th metatarsal. This may be response to a healing fracture although, do not have a history of previous 5th metatarsal injury. Please correlate with patient's clinical exam.

The 5th metatarsal and mid diaphysis does has a somewhat expansile appearance. This may simply represent a healing fracture.

Three x-rays of the left ankle demonstrated no acute fracture. The talar dome appears intact.

IMPRESSION:

1. No evidence of acute fracture involving the ankle.
2. There is some elevation of the cortex and mild expansion of the mid diaphysis of the 5th metatarsal.
3. If the patient has a history of 5th metatarsal fracture, this may represent healing response. If there is no history of 5th metatarsal fracture, then more aggressive etiology for the 5th metatarsal appearance should be considered.

Facility: LCF

Electronically signed by: Mark Moore (Feb 13, 2014 13:57:26)

Exhibit #2 - Doc. #1

Please FAX abnormal findings to:

(719) 775-7651

Limon Correctional Facility

Telephone#: (719)775-7653

Colorado Department of Corrections
X-RAY REQUEST/REPORT

CLINICAL HISTORY:

(L) foot Pain
Pain in lat metatarsal Area

UPPER EXTREMITIES			LOWER EXTREMITIES			THORAX	
Scapula	L R	73010	Pelvis		72170	Chest PA, Lat	71010
Clavicle	L R	73000	Hip	L R	73510	Chest, PPD	71000
Acro/Clav Joint	L R	73050	Femur	L R	73550	Ribs	L R 71100
Shoulder	L R	73030	Knee	L R	73564	Sternum	71120
Humerus	L R	73060	Lower Leg	L R	73592	Sternoclavicular	71130
Elbow	L R	73080	Ankle	(L) R	73600	Neck, Soft Tissue	70360
Forearm	L R	73090	Heel	L R	73650	ABDOMEN	
Wrist	L R	73110	Foot	(L) R	73630	Supine (KUB)	74000
Hand	L R	73130	Toes		73660	Supine & Upright	74022
Fingers	L R	73140	L 1 st 2 nd 3 rd 4 th 5 th			CONTRAST STUDIES	
CRANIAL BONES			R 1 st 2 nd 3 rd 4 th 5 th			Esophagram	74220
General Skull Study		70250	SPINE			Upper GI	74240
G Facial Bone Study		74140	Cervical, Ap, Lat, Obliq		72050	Barium Enema	
Orbit	L R	70190	Thoracic Study, Ap, Lat		72070	With air	74280
Mandible		70100	Lumbar Ap, Lat, Obliq		72110	Without air	74270
Tempromandibular Jt.		70328	Sacrum - Coccyx		72220	Oral Cholecystogram	74290
Nasal Bone		70130	S-I Joints		72200	IV Pyelogram	74400
Sinuses, upright Waters		70210	Scoliosis Series		72090	Sm Bowel Series	74250

OTHER:

RADIOGRAPHIC FINDINGS: (DO NOT WRITE IN BLANK AREA)

Exhibit #2 - Doc. #2

NAME Robidue, Aaron DOC # 154691

DOB 5/6/89

FACILITY LCF

REQUESTED BY: A. Massenburg PA X-RAY TECH: RF

DATE OF REQUEST: 2/4/14

DATE TAKEN: 2-5-14

35011(R 7/00) White - Medical Record Canary - X-Ray Pink - Radiologist

Request Received

Report Out



Pueblo Radiological Group
P.O. Box 9347
Pueblo, CO, 81008
T 719 584-7415
F 719 542-7019

Patient Name:	Rabidue, Aaron	Accession:	16430EDOC
MRN:	154691DOC	DoB:	May 6, 1989
Physician:	Massenburg, Alvin	Sex:	Male
Exam:	73630-FOOT, THREE VIEWS	Exam Date:	March 5, 2014 09:41
		Report Status:	Final

Findings

Reporting MD: Ballenger, Danny
Dictation Time: Not available
Transcriptionist: Mathews, Kristina
Transcription Date: March 7, 2014 16:14

*X-ray done on 3-5-14
X-ray received 3-11-14*

LEFT FOOT, THREE VIEWS

HISTORY: Injury and pain.

COMPARISON: 02/05/2014.

FINDINGS: Normal alignment to the bones of the foot. A cortical defect through the distal diaphysis of the 2nd metatarsal consistent with an acute fracture. This is not identified on the previous exam. There is a healing fracture through the proximal diaphysis of the 4th metatarsal. An old fracture, which appears healed through the mid diaphysis of the 5th metatarsal.

IMPRESSION: Series of fractures with suggestion of different stages of healing. An acute fracture of the 2nd metatarsal. A subacute-chronic fracture of the proximal 4th metatarsal and a chronic fracture of the 5th metatarsal. Alignment anatomic.

Facility: LCF

Electronically signed by: Danny Ballenger (Mar 10, 2014 16:30:25)

Exhibit #2 - Doc. #3